A Well Child Visit (EPSDT)

The purpose of EPSDT services is:

- To actively seek out all eligible families and educate them on the benefits of preventive health care
- To help recipients effectively use health resources and encourage them to participate in the screening program at regular intervals
- To provide for the detection of any physical and mental problems in children and youth as early as possible through comprehensive medical screenings in accordance with program standards
- To provide for appropriate and timely services to correct or ameliorate any acute or chronic conditions

This appendix offers information about the EPSDT program. It consists of the following sections:

Section	***Contents***
Understanding EPSDT	Provides an overview of EPSDT, including descriptions of screening types and services offered under EPSDT
Performing Screenings	Provides information on becoming an EPSDT screening provider, verifying recipient eligibility, critical components of screenings, and how to submit claims for EPSDT screenings
Providing and Obtaining Referrals	Describes the process for providing referrals to specialists and obtaining referrals from screening providers. Please refer to Section A.4: Providing and Obtaining Referrals, for more information.
Coordinating Care	Describes the administrative requirements of the EPSDT program, including consent forms and retention of medical records.
Justice Involved Youth (JIY) Travel Payment	Provides information on the JIY Travel Bonus Payment and related billing and reimbursement guidelines. See Section A.5 for further details.
Off-site Screenings	Provides an overview of the off-site screening program, including enrollment requirements, components required, eligibility verification, referral process and reimbursement information.
Vaccines for Children	Describes the Vaccines for Children program, including enrollment instructions, which procedure codes to bill, how to bill for administration fees, and a copy of the immunization schedule.

Added: Justice Involved Youth…for further details.

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A.1 Understanding EPSDT

The purpose of the EPSDT program is to find children with actual or potential health problems and to screen, diagnose, and treat the problems before they become permanent, lifelong disabilities. The program also offers preventive health services to Medicaid-eligible children under 21 years of age.

The EPSDT services are available up to 30 days prior to release for an eligible child under age 21 being held post adjudication as an inmate of a public institution. (Adjudication is the court process that determines if an individual committed the act for which they were charged.

The EPSDT program was expanded in the Omnibus Budget Reconciliation Act of 1989 to allow additional services. The acronym EPSDT stands for:

Early	A Medicaid-eligible child should begin to receive high quality preventive health care as early as possible in life.
Periodic	Preventive health care occurring at regular intervals according to an established schedule that meets reasonable standards of medical, vision, hearing, and dental practice established by recognized professional organization.
Screening	A comprehensive, unclothed head-to-toe physical examination to identify those who may need further diagnosis, evaluation, and/or treatment of their physical and mental problems.
Diagnosis	The determination of the nature or cause of physical or mental disease, conditions, or abnormalities identified during a screening.
Treatment	Any type of health care or other measures provided to correct or ameliorate defects, physical and mental illnesses, or chronic conditions identified during a screening.

Periodicity Schedule

Periodic screenings must be performed in accordance with the schedule listed below. This schedule is based upon the recommendations of the American Academy of Pediatrics.

- 3-5 day old
- 1 month
- 2 months
- 4 months
- 6 months
- 9 months
- 12 months
- 15 months
- 18 months
- 24 months
- 30 months
- Then annually (per calendar year) through 20 years of age beginning with the third birthday

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NOTE:

Medicaid will reimburse for only one screening per calendar year for children over the age of three. Screening benefit availability may be verified through AVRS, Gainwell Provider Electronic Solutions software, or the Provider Assistance Center at Gainwell. Please refer to Chapter 3, Verifying Recipient Eligibility, for more information.

If a periodic screening has not been performed on time according to the periodicity schedule (for instance, if the 2 months' periodic screening was missed), a screening may be performed at an “in between” age (for example, at 3 months) and billed as a periodic screening. In other words, the child should be brought up to date on his/her screening according to his/her age. Re-screenings should occur within 2 weeks (before or after) of the established periodicity schedule. This policy applies to recipients 0-24 months of age.

EPSDT screenings fall under six broad categories:

Type of Screening	***Description***
Initial Screening	Initial screenings indicate the first time an EPSDT screening is performed on a recipient by an EPSDT screening provider.
Periodic Screening	Periodic screenings are well child visits performed based on a periodicity schedule. The ages to be screened are 3-5 days, 1 month, 2 months, 4 months, 6 months, 9 months, 12 months, 15 months, 18 months, 24 months, 30 months, and annually beginning on or after the child’s third birthday.
Interperiodic Screening	Interperiodic screenings are considered problem-focused and abnormal. These are performed when medically necessary for undiagnosed conditions outside the established periodicity schedule and can occur at any age.
Vision Screening	Vision screenings must be performed on children from birth through age two by observation (subjective) and history. Objective testing begins at age three and should be documented in objective measurements.
Hearing Screening	Hearing screenings must be performed on children from birth through age four by observation (subjective) and history. Objective testing begins at age five and should be recorded in decibels.
Dental Screening	Dental screenings must be performed on children from birth through age two by observation (subjective) and history. Beginning with age one, recipients must be either under the care of a dentist or referred to a dentist for dental care.
Emotional and behavioral Screening	Emotional Assessment will cover procedure code 96127 for children age 3 months - 20 years. Assessment must be ordered and signed by a physician or Non- Physician Practitioner (i.e., Psychologist, Physician Assistant, or Certified Registered Nurse Practitioner).

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A.2 Using PT+3 with EPSDT services

A patient education method (PT+3) has recently been developed for working with illiterate or marginally literate individuals. The PT+3 allows providers to make the most of patient contacts as opportunities to provide developmentally appropriate information for recipients and their families.

The acronym PT+3 means:

P = Personalize the problem

T = "TAKLE" the problem:

- T = set a Therapeutic Tone,
- A = Assess the knowledge level of the patient,
- K = provide Knowledge,
- L = Listen for feedback,
- E = Elaborate or reeducate as needed.

+3 = Summarize the teaching session into three essential points.

PT+3 is a standardized protocol that provides the skills and structure for health care providers to assist young or marginally literate patients in learning and remembering essential points from a health care encounter. PT+3 is designed to increase patient knowledge and compliance. Patients seem to like and understand the simplified information and providers like the process. Using PT+3 saves time for providers and enhances the medical visit for the recipient. PT+3 enables individuals to remember the most important aspects of the medical visit.

Specially designed low literacy materials are available for children (EPSDT Brochures), teens ("How to Talk to Your Children"), and adults ("Facts about Birth Control") and are free to providers including EPSDT, ACHN, and Medicaid family planning providers who receive training in the use of the PT+3 method of education. For more information regarding PT+3, please fax your request to (334) 353-5203, attention "Outreach & Education." Please include your name and telephone number.

A.3 Performing Screenings

This section describes becoming an EPSDT screening provider, verifying recipient eligibility, scheduling screenings, critical components of screenings, and submitting claims for EPSDT screenings.

A.3.1 Becoming an EPSDT Screening Provider

Participation as an EPSDT screening provider is voluntary. To become an EPSDT screening provider, a provider must be an approved Alabama Medicaid provider and must have a 10-digit NPI. New providers should refer to Chapter 2, Becoming a Medicaid Provider, for instructions on receiving an application.

Federal requirements mandate providers re-validate periodically with the Alabama Medicaid program. Providers will be notified when they are scheduled to re-validate. Failure to re-validate and provide appropriate documentation to complete enrollment will result in an end-date being placed on the provider file. Once a provider file has been closed for failure to timely re-validate, providers will have to submit a new application for enrollment.

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Current Medicaid providers who wish to become an EPSDT screening provider may download the information and forms from the Alabama Medicaid website (https://medicaid.alabama.gov/) or contact the Gainwell Provider Enrollment Unit at the following address:

Gainwell Provider Enrollment
P.O. Box 241685
Montgomery, Alabama 36124-1685
1 (888) 223-3630

Provider Types Eligible for Participation

Only certain Alabama Medicaid provider types may become approved EPSDT screening providers. In some cases, these providers are restricted to where they can perform screenings:

This Provider Type	***May Perform Screenings at the Following Locations:***
Physicians	Anywhere a physician is authorized to practice
Nurse practitioners	At a physician's office, Rural Health Clinic, Federally Qualified Health Care Clinic (FQHC), health department, or hospital
Registered Nurses	At a rural health clinic, Federally Qualified Health Care Clinic (FQHC), health department, or hospital **NOTE:** Two-year degree RNs who wish to perform EPSDT screenings must first complete a Medicaid-approved pediatric health assessment course (PAC) or show proof of completion of a similar program of study. BSN's are exempt from taking a PAC.
Physician Assistants	At a physician's office, rural health clinic, Federally Qualified Health Care Clinic (FQHC), health department, or hospital

Providers are not limited to those who are qualified to provide the full range of medical, vision, hearing, and dental screening services. Although a qualified provider may be enrolled to furnish one or more types of screening services, the Alabama Medicaid Agency encourages qualified providers to provide the full range of medical, vision, hearing, and dental screening services to avoid fragmentation and duplication of services.

NOTE:

Medical screenings, including the physical, must be performed by a physician, certified nurse practitioner, registered nurse, or physician's assistant, who is approved to perform well child visits. Other trained personnel may perform some screening components (for instance, measurements or finger sticks).

Refer to Chapter 112, Telemedicine Service, for general benefit information and limitations.

Potential EPSDT off-site providers must submit specific documents (see Section A.5) and be approved to participate as an off-site provider.

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A.3.2 Verifying Recipient Eligibility

Reimbursement will be made only for eligible Medicaid recipients. Eligibility and benefit limits should be verified **prior to rendering** services to **ANY** Medicaid recipient.

NOTE:

Every effort should be made to assure that medical, dental, vision, and hearing screenings, including immunizations, are accomplished in one visit, and that fragmentation or duplication of screening services is prevented. Section A.7, Vaccines for Children, describes the immunization schedule.

To be eligible for Medicaid, recipients must meet certain criteria. It is crucial for a provider to check the eligibility prior to performing services. Eligibility verification will help ensure that the provider claim gets paid. Some examples of what a provider will be informed of when checking eligibility include:

- The type of eligibility which indicates services the recipient may obtain
- The attributed provider
- The number of screenings that have been performed
- The 13th digit of the recipient Medicaid identification number

For details about eligibility please refer to Chapter 3: Verifying Recipient Eligibility.

A.3.3 Outreach

Outreach activities are critical to successful health screening services. The outreach process assures that eligible families are contacted, informed, and assisted in securing health-screening services.

The Alabama Medicaid Agency, in conjunction with the Department of Human Resources, informs the applicant of EPSDT services. SSI (Category 4) eligible recipients are informed of EPSDT services. The Medicaid-eligible child is permitted to see any Alabama Medicaid provider for EPSDT services without a referral from PCP.

Once the child has had an EPSDT screening, all subsequent visits to other providers for further diagnosis or treatment must have a prior approved written referral (Form 362) from the EPSDT screening provider.

For more information regarding the ACHN, refer to Chapter 40 - ACHN of this manual or call the Provider Assistance Center at (800) 688-7989.

The Alabama Medicaid Agency's goal is to provide effective outreach services for Medicaid-eligible recipients. EPSDT outreach efforts are aimed at two groups: (a) new Medicaid recipients and (b) all Medicaid-eligible recipients under 21 years of age who have not had a well child screening in the last 12 months. These recipients are notified twice annually. The recipient is informed about EPSDT services through an outreach letter and is encouraged to make an appointment for an EPSDT screening.

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A.3.4 EPSDT Care Coordination

The goal for ACHN Care Coordination Services is to provide children with opportunities to maximize their health and development by ensuring the availability and accessibility of comprehensive and continuous preventive health services throughout childhood. The ACHN Care Coordination services are available to any provider, at no cost, who wishes to benefit from effective intervention. These Medicaid eligible recipients will then be targeted for outreach.

The scope of services are also designed to support and assist your office personnel with identifying, contacting, coordinating, and providing follow up office visits for children who are behind on their EPSDT screenings. These services include but are not limited to: immunizations, vision/hearing screenings, dental screenings, identifying recipients who have high utilization of emergency room visits; follow up regarding referrals, missed appointments, and coordination for teen pregnancy prevention services. In addition, Care Coordinators are available to assist with transportation services using Alabama Medicaid's Non-Emergency Transportation (NET) program. ACHN Care Coordinators may receive referrals from physicians and dentists regarding medically-at-risk clients who need assistance with keeping appointments and obtaining follow-up care. Lastly, ACHN Care Coordinators will encourage and assist in recruiting private physicians to improve services to this population.

Participation of qualified EPSDT Care Coordination services is available through the ACHNs whose primary role is that of care coordinator. Active physician involvement for treatment is vital. ACHN Care Coordination services are available by contacting your regional ACHN. Please visit our website at www.medicaid.alabama.gov and select "ACHN": ACHN Home: ACHN Providers: "ACHN Interactive Map" or ACHN Contact Information for Providers. A list of ACHN Care Coordinators by county and telephone numbers is available to support your office personnel.

A.3.5 Scheduling Screenings

The Alabama Medicaid Agency requires that persons requesting screening services receive the services within 120-180 days from the date the request was made. These persons should be given priority by the screening agency when scheduling appointments.

EPSDT selected providers and Primary Care Physician (PCP) receive a periodic re-screen list each month. The provider should utilize the periodic re-screen list to notify the EPSDT-eligible recipient when the medical screening is due. An appointment should be made for the next screening on the periodicity schedule. These functions are an integral part of the full screening provider's responsibility and are essential for care coordination. Providers have a total of 120 days from due date or award date (listed on printout) to accomplish screening, necessary referral, and treatment for the recipients listed on the printout.

EPSDT-eligible Medicaid beneficiaries who request well child visits must be provided regularly scheduled examinations and assessments at the intervals established by Medicaid policy.

Scheduling of initial and periodic screenings is the responsibility of the screening provider. The ACHN is responsible for overall care coordination for medical, vision, hearing, and dental screenings for recipients who participate in the ACHN program.

The EPSDT screening provider should not perform a screening if written verification exists or if notified by another provider that the child has received the most recent age

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appropriate screening. An additional interperiodic screening may be performed if requested by the parent or if medically necessary.

Please refer to Section A.5, Care Coordination, for more information on screening provider responsibilities.

A.3.6 Critical Components of Screenings

This section describes critical components of periodic, interperiodic, and vision/hearing/dental screenings. It also describes recommended health education counseling topics by age group.

Periodic Screenings

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Component	*Description*
Unclothed physical exam	This is a comprehensive head-to-toe assessment that must be completed at each screening visit and include at least the following: • Temperature, and height/weight ratio • Head circumference through age two • Blood pressure and pulse at age three and above • Measure body-mass index when clinically indicated Body-mass index (BMI) – BMI should be performed at each visit if clinically indicated. BMI-for-age charts are recommended to assess weight in relation to stature for children ages 2 to 20 years. The weight-for-stature charts are available as an alternative to accommodate children ages 2-5 years who are not evaluated beyond the preschool years. However, all health care providers should consider using the BMI-for-age charts to be consistent with current recommendations. The charts are available on the American Academy of Pediatrics website at http://www.aap.org.
Comprehensive family/medical history	This information must be obtained at the initial screening visit from the parent(s), guardian, or responsible adult who is familiar with the child's history. The history must include an assessment of both physical and mental health development and the history must be updated at each subsequent visit.
Immunization status	Immunizations and applicable records must be updated according to the current immunization schedule of the Advisory committee on Immunization Practices (ACIP). Dates and providers must be recorded in the medical record indicating when and who gave the vaccines, if not given by the screening provider. The state law has been changed so that private and public healthcare providers may share immunization data. Medicaid recipients shall be deemed to have given their consent to the release by the state Medicaid Agency of information to the State Board of Health or any other health care provider, by virtue immunization data should be recorded in the medical record.
Vaccine and Immunization Counseling	Vaccine and immunization counseling is a recommended component of each periodic EPSDT/well-child screening visit. Counseling must be provided to the parent/guardian and/or age-appropriate recipient at each periodic screening visit. This includes discussion of the recommended ACIP immunization schedule, benefits, risks, and addressing questions or concerns. Counseling may be billed using the following codes: • **90460/90461** – Vaccine administration counseling (with or without vaccine administration) when provided during a well-child (EPSDT) visit • **99401** – Preventive medicine counseling (approximately 15 minutes) as part of a well-child visit when it includes vaccine counseling along with other anticipatory guidance or preventive topics • **G0312** – stand-alone immunization counseling (general), 5-15 minutes (no vaccine given same day) • **G0313** – stand-alone immunization counseling (general), 16-30 minutes (no vaccine given same day) • **G0314** – stand-alone immunization counseling for COVID-19, 16-30 minutes (no vaccine given same day) • **G0315** – stand-alone immunization counseling for COVID-19, 5-15 minutes (no vaccine given same day) Append modifier **EP** to all counseling codes to identify the service as part of the EPSDT benefit. When counseling is a significant, separately identifiable service on the same day as the well-child visit E/M code, append modifier **25** to the E/M code. Important Billing Note: Only one vaccine or preventive counseling code may be billed per date of service. Providers may not bill combinations with **99401**, **90460/90461**, **G0312**, **G0313, G0314, or G0315** on the same claim.

Added: Vaccine and Immunization…the same claim.

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TB skin test	Children who should be considered for tuberculin skin testing (TST): 1. Children whose parents (with unknown TST status) immigrated from regions of the world with high prevalence of tuberculosis; continued potential exposure by travel to endemic areas and/or household contact with persons from the endemic areas (with unknown TST status) should be tested. 2. Children without specific risk factors who reside in high-prevalence areas should be tested. In general, a high-risk neighborhood or community does not mean an entire city is at high risk. Rates in any area of the city may vary by neighborhood or even from block to block. Physicians should be aware of these patterns in determining the likelihood of exposure; public health officials or local tuberculosis experts should help physicians identify areas with appreciable tuberculosis rates. 3. Children at increased risk for progression of infection to disease should be tested. Those with medical conditions such as diabetes mellitus, chronic renal failure, malnutrition or congenital or acquired immunodeficiency deserve special consideration. Without recent exposure, these children are not at increased risk of acquiring tuberculosis infection. Underlying immune deficiencies associated with these conditions theoretically enhance the possibility for progression to severe disease. Initial histories of potential exposure to tuberculosis should be included for all of these patients. If these histories or local epidemiologic factors suggest a possibility of exposure, immediate and periodic TST should be considered. An initial TST should be performed before initiation of immunosuppressive therapy for any child with an underlying condition that necessitates immunosuppressive therapy. Bacille Calmette-Guérin (BCG) immunization does not induce positive results when TB blood tests are used. TB blood tests are preferred for people who have received the BCG vaccine, including children. HIV indicates human immunodeficiency virus.

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Definitions of positive TST	Definitions of positive tuberculin skin testing (TST) per CDC.gov: 1. TST should be read between 48 - 72 hours after administration by a health care worker trained to read TST results. 2. The diagnosis of TB disease in children is often made without laboratory confirmation and instead based on combination of the following factors: • Clinical signs and symptoms typically associated with TB disease, • Positive tuberculin skin test (TST) or positive TB blood test (IGRA), • Chest x-ray that has patterns typically associated with TB disease, and • History of contact with a person with infectious TB disease. 3. Classification of the TST Reaction **A. Induration >5mm** • People living with HIV. • A recent contact of a person with infectious TB disease. • People with chest x-ray findings suggestive of previous TB disease. • People with organ transplants. • Other immunosuppressed people (e.g., patients on prolonged therapy with corticosteroids equivalent to/greater than 15 mg per day of prednisone or those taking TNF-a antagonists) **B. Induration >10mm** • People born in countries where TB disease is common, including some countries in Asia, Africa, and Latin America • People who misuse drugs and alcohol • People who live or work in high-risk congregate settings (e.g.' nursing homes, homeless shelters, or correctional facilities) • People with certain medical conditions that place them at high risk for TB (e.g., silicosis, diabetes mellitus, severe kidney disease, certain types of cancer, and certain intestinal conditions) • People with low body weight (<90% of ideal body weight) • Children younger than 5 years of age • Infants, children, and adolescents exposed to adults in high-risk categories **C. Induration >15mm** People with no known risk factors for TB
Developmental surveillance and assessment	A comprehensive developmental history is required, if appropriate, to determine the existence of motor, speech, language, and physical problems or to detect the presence of any developmental lags. An age-appropriate developmental assessment is required at each screening. Information must be acquired on the child's usual functioning as reported by the child's parent, teacher, health care professional, or other knowledgeable individual. **Developmental assessments must be performed by a RN, BSN; CRNP, PA, or Physician.**
Nutritional status screening	Nutritional status must be assessed at each screening visit. Screenings are based on dietary history, physical observation, height, weight, head circumference (ages two and under), hemoglobin/hematocrit, and any other laboratory determinations carried out in the screening process. A plotted height/weight graph chart is acceptable when performed in conjunction with a hemoglobin or hematocrit if the recipient falls between the 10th and 95th percentile.
Health education including anticipatory guidance	Health education and counseling for parent(s) or guardian and the youth (if age appropriate) are required at each screening visit. Health education is designed to assist the parent in understanding what to expect in terms of development. Health education also provides information about the benefits of healthy lifestyles and practices, as well as accident and disease prevention. Providers may use the PT+3 teaching method for anticipatory guidance counseling. PT+3 should be documented in the medical record (i.e., progress notes) listing the three points emphasized.
Objective Developmental Screenings	SEE BELOW

Deleted: ~~M.D.~~
Added: Physician

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Objective Developmental Screenings

EPSDT providers are allowed to bill for an objective developmental screening in addition to an EPSDT screening at the 9-month, 18-month, and 30-month well-child visit. EPSDT providers also have the option of providing the developmental screening anytime that surveillance (medical history of developmental risk factors, parental/caregiver concern) identifies a need. Providers are encouraged to use standardized screening tools that have a moderate to high sensitivity, specificity, and validity level and are culturally sensitive. The following code, which is limited to five (5) units per date of service (five different screening tools used), may be used to bill for this screening:

> 96110 - Developmental testing; limited (e.g., Developmental Screening Test II, Early Language Milestone Screen), with interpretation scoring and reporting documentation per standardized instrument (provider must document description of score).

In order to bill this code, providers must use a standardized screening tool. Examples of screening tools allowed for this code include, but are not limited to:

- Ages and Stages Questionnaire (ASQ)
- Denver DST/Denver II
- Battelle Developmental Screener
- Bayley Infant Neurodevelopment Screener (BINS)
- Parents Evaluation of Development (PEDS)
- Early Language Accomplishment Profile (ELAP)
- Brigance Screens II
- Pediatric Symptom Checklist (PSC)

The developmental screening tool must be ordered by the provider or on behalf of the provider based on developmental screening written protocol/standing orders and scored (Provider must document description of score). The provider's physical or electronic signature must be in the medical record. Provider includes physician or non-physician practitioner, (i.e., Psychologist, Physician Assistant, Certified Registered Nurse Practitioner, and Certified Pediatric Nurse Practitioner). If a standardized tool has a summary form, the summary must be retained/scanned into the medical record. For any other form with no summary, retain/scan the full document.

Objective Developmental Testing

Objective developmental testing is when a physician or other health care professional measures cognitive, motor, social, language, adaptive, and/or cognitive abilities using provider standardized tests (meeting industry standards) via written, oral, or combined format testing. The following codes apply to testing for developmental disorders for recipients up to 20 years old and must include the interpretation and report on the findings.

A. Procedure Codes:

1. 96112 – Developmental test administration (including assessment of fine and/or gross motor, language, cognitive level, social, memory and/or executive functions by standardized developmental instruments) when performed by physician or other qualified healthcare professional, with interpretation and report; first hour

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- Report 96112 for the initial hour of testing, with interpretation and report.
- 96112 may not be reported for less than 31 minutes of time.

2. 96113 – Each additional 30 minutes (list separately in addition to code 96112)

- Report 96113 for each additional 30 minutes, with interpretation and report.
- 96113 may not be reported for less than 16 minutes.

B. Documentation Requirement:

1. Generally, the documentation of objective developmental testing will include: scoring, interpretation, and preparation of a separate identifiable report.

- The report must include the following: identifying data, date/time and location of testing, reason for the type of testing being done, and titles of all instruments completed by the child.
- The report must also include the test protocol, all scoring sheets, and the scored item results.
- The provider's interpretation and plan need to be noted in the report.

2. An appropriate provider signature and date is required according to Alabama Administrative Code Rule 560-X-1-.18.

3. The total time spent on these services for the patient is required. If time is not documented, the code(s) may not be reported.

- If the testing is performed over several days, the time for all testing should be combined and reported on the last day of service.

C. These services will be subject to post-payment review.

Brief Emotional and Behavioral Assessment

EPSDT providers are allowed to bill for an emotional and behavioral assessment in addition to an EPSDT screening for ages 3-5 days to 20 years. EPSDT providers also have the option of providing the emotional and behavioral assessment anytime that surveillance (medical history of risk factors, parental/caregiver concern) identifies a need. Providers are encouraged to use standardized tools that have a moderate to high sensitivity, specificity and validity level and are culturally sensitive. The following code, which is limited to three (3) units per date of service (three different screening tools used), may be used to bill for this screening:

> 96127 – Brief emotional/behavioral assessment (e.g., depression inventory, attention-deficit/hyperactivity disorder [ADHD] scale), with scoring and documentation, per standardized instrument (Provider must document description of score).

Examples of screening tools allowed for this code include, but not limited to:

- Ages and Stages Questionnaire/ Social Emotional (ASQ-SE)
- Modified Checklist for Autism in Toddlers (M-CHAT)
- NICHQ Vanderbilt Assessment Scales
- Behavior Assessment Scale for Children Second
- Patient Health Questionnaire (PHQ – 2 AND PHQ – 9)

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- Pediatric Symptom Checklist (PSC)

The emotional and behavioral assessments must be ordered by the provider or on behalf of the provider based on emotional and behavioral assessment written protocol/standing orders and scored Provider must document description of score). The provider's physical or electronic signature must be in the medical record. Provider includes physician or non-physician practitioner, (i.e., Psychologist, Physician Assistant, Certified Registered Nurse Practitioner, and Certified Pediatric Nurse Practitioner). If a standardized tool has a summary form, the summary must be retained/scanned into the medical record. For any other form with no summary, retain/scan the full document.

Vision Testing/Screenings

Vision screenings are available either as a result of the EPSDT referral or as a result of a request/need by the recipient. A subjective screening for visual problems must be performed on children from birth through age two by history and observation. Gross examinations should be documented as grossly normal or abnormal. Objective testing begins at age three. Visual acuity screening must be performed through the use of the Snellen test, Allen Cards, photo refraction, or their equivalent. Objective testing must be referred out if not performed by the screening provider.

If a child is uncooperative, perform a subjective assessment. The reason(s) for not being able to perform the test must be documented in the medical record. Proceed with billing the vision screening on the same date of service as the initial or periodic screening. The child should be rescheduled for an appointment to complete the vision screening. Be sure to complete the vision screening within 30-45 days from the original screening date.

If a suspected visual problem manifests itself, regardless of whether such services coincide with the periodicity schedule, an interperiodic screening should be scheduled with the child's physician so the history and problem-focused physical exam, can be obtained and an EPSDT referral issued to the appropriate specialist or consultant.

Providers **must** use an "EP" modifier to designate all services related to EPSDT well-child visits, **including routine vision** and **hearing screenings**. Post payment reviews are performed to determine appropriate utilization of services.

Trained office staff may perform a vision screening if successfully trained. A staff member must meet the following criteria to be considered trained.

- Employee observes a vision screening being performed on a minimum of three patients by a skilled/trained employee
- Employee verbalizes an understanding of the steps required to perform a vision screening
- Employee performs a vision screening under supervision on a minimum of three patients successfully.

Hearing Testing/Screenings

Hearing screenings are available either as a result of an EPSDT referral or as a result of a request/need by the recipient. A subjective screening for hearing problems must be performed on children from birth through age four by history and observation. Gross examination should be documented as grossly normal or abnormal. Objective testing begins at age five. Hearing screenings must be performed through the use of a pure tone audiometer at 500 and 4,000 Hz at 25 decibels for both ears. If a child fails

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to respond at either frequency in either ear, a complete audiogram must be done. Objective testing must be referred out if not performed by the screening provider.

If a child is uncooperative, do a subjective assessment. The reason(s) for not being able to complete the test must be documented in the medical record. Proceed with billing the hearing screening on the same date of service as the initial or periodic screening. The child should be rescheduled for an appointment to complete the hearing screening. Be sure to complete the hearing screening within 30-45 days from the original screening date.

If a suspected hearing problem manifests itself, regardless of whether such services coincide with the periodicity schedule, an interperiodic screening should be scheduled with the child's physician so the history and problem-focused physical exam can be obtained and an EPSDT referral issued to the appropriate specialist or consultant.

Trained office staff may perform a hearing screening if successfully trained. A staff member must meet the following criteria to be considered trained.

- Employee observes a hearing screening being performed on a minimum of three patients by a skilled/trained employee
- Employee verbalizes an understanding of the steps required to perform a hearing screening
- Employee performs a hearing screening under supervision on a minimum of three patients successfully.

EP modifiers are used when billing initial, periodic and interperiodic screenings. Once chronic problems have been identified, providers should bill the appropriate level of office visit code and bill the services as EPSDT referring.

Dental Services

Dental care is limited to Medicaid-eligible individuals who are eligible for treatment under the EPSDT Program. Dental screenings must be performed on children from birth through age two by observation/inspection and history. Beginning with age one, recipients must be either under the care of a dentist or referred to a dentist for dental care.

A periodic oral examination is recommended once every six months for eligible Medicaid recipients under 21 years of age. Dental services include emergency, preventive, and therapeutic services as well as orthodontic treatment when medically necessary. A referral, or documentation that recipient is under the care of a dentist is required at age one and older. Follow-up is no longer mandatory. Any time a need for dental care is identified, regardless of the child’s age, the child should be referred to a dentist.

Beginning with age one, providers should educate and document that caretakers have been advised of the importance (anticipatory guidance) of good oral healthcare and the need to make a dental appointment. Additional documentation suggestions include providing the caretaker with one of the following: www.insurekidsnow.gov, or the Recipient Hotline number to assist with locating a dentist (800) 362-1504.

Effective 01/01/2009, Pediatric providers (MDs, DOs, PAs, and NPs) are be able to bill in accordance with Medicaid reimbursement policies for oral assessment and application of fluoride varnish for recipients age 6 months through 35 months of age. Providers may bill assessment code D0145 (oral exam less than 3 years old, counseling with primary caregiver). D0145 can be billed once by pediatric medical provider and once by the dental provider for recipients 6 months through 35 months of

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age. Varnishing code D1206 (topical fluoride application) will be limited to 3 per calendar year, not to exceed a maximum of 6 fluoride varnish applications between 6 months and 35 months of age. Frequency is not allowed less than 90 days. Providers are required to bill ICD 10 diagnosis code Z13.84 or Z29.3 on or after October 01, 2015.Once a recipient has an established dental home (described in Chapter 13), a pediatrician cannot bill D1206.

Dental care under the Program is available either as a result of the EPSDT referral or as a result of request/need by the recipient. Conditions for each situation are as follows:

1. EPSDT Referral – If the EPSDT Screening Provider determines a recipient requires dental care or if the recipient is age one or older and is not currently under the care of a dentist, the recipient must be referred to an enrolled dentist for diagnosis and treatment. After the recipient's dental care is initiated, the consultant's portion of the Referral Form (Form 362) must be completed by the dentist and the appropriate copy must be returned to the screening provider.
2. Recipient Seeking Treatment – If a recipient who has not been screened through the EPSDT Program requires dental care, care may be provided without having a Referral Form. Dental care provided on request of the recipient is considered a partial screening. In this situation, after the required care is completed, the dentist should advise the recipient to seek an EPSDT screening provider to obtain a complete medical assessment.

NOTE:

Dental health care services are available for eligible children under age 21, as part of the EPSDT program. To obtain information about dentists. Please contact Provider Assistance Center at (800) 688-7989.

Laboratory Screenings

Laboratory screening procedures must be performed in coordination with other medical screening services at the same visit, whenever possible. If verifiable results are available from another provider that any required laboratory procedure was performed within 30 days prior to the screening visit and there is no indication of a diagnosis that would warrant that the test be redone, it is not necessary to perform the test again. However, the test results or a copy of the test results should be documented in the medical record.

NOTE:

Physicians can now draw lead and hgb and/or hct at the 12-month visit.

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The following is a list of tests and procedures of laboratory screenings:

Laboratory Test	*Description*
Newborn Screening	Newborn screening is mandated by **Statutory Authority Code of Alabama 1975, Section 22-20-3**. Every hospital or facility providing delivery services is required to screen all infants for these potentially devastating disorders. The Alabama Department of Public Health (ADPH) is responsible for administrative oversight of the Newborn Screening Program. Infants are screened for 32 primary and 45 secondary disorders which include Endocrine Disorders (Congenital Hypothyroidism and Congenital Adrenal Hyperplasia), Cystic Fibrosis (CF), Sickle Cell Disease, Hearing Loss, Metabolic Disorders (Amino, Fatty, and Organic Acid), severe combined immunodeficiency (SCID), spinal muscular atrophy (SMA), and critical congenital heart disease through pulse oximetry screening. The Alabama Newborn Screening website has a complete list of disorders at www.alabamapublichealth.gov/newbornscreening. Early diagnosis of these conditions may reduce morbidity, premature death, intellectual disability, and other developmental disabilities. All initial screening tests are conducted by ADPH's Bureau of Clinical Laboratories (BCL). Infants 12 months of age and younger with no record of a newborn screen should be tested as soon as possible. Screening for hemoglobinopathies (sickle cell disease/trait or thalassemia) is only included on the initial newborn screen. If initial results are not satisfactory for infants from birth to 12 months of age a repeat test must be performed. Children over 12 months of age who have never been tested need only be screened when ordered by a physician. ADPH's BCL has established standards and cutoffs for newborns and infants, and therefore, **cannot accept specimens on infants older than 12 months of age**. Please see the Newborn Screening Collection Guidelines at https://www.alabamapublichealth.gov/newbornscreening/assets/newbornscreeningbloodcollectionguidelines.pdf Routine second testing is recommended between two and six weeks of age, with four weeks being optimal. This second screen is critical in detecting a condition that may not have been picked up on an initial screen. Confirmation of abnormal newborn screening results is always necessary. An infant with a positive screen should be referred for diagnostic testing. The Alabama Newborn Screening Program works in partnership with pediatric sub-specialists to ensure all babies identified with abnormal results receive appropriate follow-up. These specialists are located at the University of Alabama in Birmingham (UAB), University of South Alabama in Mobile, and the St. Jude Clinic in Huntsville. In addition, there are seven community-based Sickle Cell Organizations who provide counseling and follow-up for infants identified with sickle cell disease and trait. The Cystic Fibrosis Care Center at UAB provides CF care to include genetic counseling at Children's of Alabama. It should be noted physicians should not bill for laboratory tests performed by the BCL. However, procedure codes 36415 and 36416 with modifier 90 may be billed for the specimen collection when referred to an outside laboratory. All screening tests are conducted by Alabama Department of Public Health's Bureau of Clinical Laboratories.

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Laboratory Test	*Description*
Public Health: Secure Remote Viewer (SRV)	The Secure Remote Viewer (SRV) is a web-based system that allows healthcare providers access to newborn screening results. The system allows users to search, view, and print results immediately from their computer. SRV is offered by the Alabama Department of Public Health, Bureau of Clinical Laboratories (BCL). The SRV provides 24-hour, seven days a week web-based reporting of screening results. The SRV requires registration with the BCL. Physicians may register with the system by completing the registration form found at https://www.alabamapublichealth.gov/newbornscreening/assets/srv_registration_form.pdf. Each physician is required to provide his/her state license number, national provider identifier (NPI), and an email address. On the registration form, physicians are asked to provide three options for the account's username. Once registration is complete, the registrant will receive a username and password via the email account provided. The email will not include the link to the SRV website for security purposes. For additional questions or information, visit https://www.alabamapublichealth.gov/newbornscreening/assets/srvregistration-instructions_lab_10-23-2023.pdf or call the BCL at (334) 290-3097.
Iron Deficiency Anemia Screening	Hematocrit and/or hemoglobin values must be determined at a medical screening visit by 12 months of age. Hematocrit and/or hemoglobin must also be determined between 11-20 years of age. Any other time must be deemed medically necessary based on physical examination and nutritional assessment.
Urine screening	Effective 10/01/2008 the urinalysis component of an EPSDT screening is no longer a requirement. A urinalysis should only be performed if clinically indicated. If a urinalysis is needed, the required screening procedure is a dipstick that shows the measurement of protein and glucose. Urine obtained from recipients between 11 and 20 years of age should also be checked for leukocytes.
Chlamydia Screening	Chlamydia Screening is recommended for all sexually-active females aged <25 years annually.

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NOTE:

The hgb or hct are included in the screening reimbursement and should not be billed separately.

Laboratory Test	*Description*
Lead toxicity screening	All children must have a blood lead toxicity screening at 12 and 24 months of age. A lead toxicity screening is also required for any child 24 to 72 months of age who has not previously received a blood lead toxicity screening or who presents with symptoms of possible lead poisoning. All children should receive lead toxicity screenings since all children are vulnerable to blood lead poisoning. Children's blood lead levels increase most rapidly at 9-12 months of age and peak at 18-24 months of age. The screening test of choice is blood lead measurement (replaces the erythrocyte protoporphyrin (EP) test.
Other lab tests	There are several other tests to consider in addition to those listed above. Their appropriateness is determined by an individual's age, sex, health history, clinical symptoms, and exposure to disease. These may include, for example, a pinworm slide, urine culture, VDRL, GC cultures and stool specimen for parasites, ova, and blood. **Note:** The test for VDRL, gonorrhea cultures, intestinal parasites, and pinworms may be done by the Alabama Department of Public Health clinical laboratory at NO cost to the EPSDT screening provider. The State lab slip must have "EPSDT Program" documented across the top. Other Medicaid approved laboratories may be used to run sickle cell and lead screening tests.

NOTE:

The State Laboratory will supply microvettes, mailing containers and forms for obtaining blood lead levels at no cost to providers upon request. Please contact (334) 290-6130 to obtain additional information.

Public Health Department Services

EPSDT care coordination is initiated for children with a confirmed blood lead level of > 3.5 μg/dL. Case management services may be requested if the physician determines the family requires additional education in the home. A physician's order is required. EPSDT care coordinators assess the family's social and environmental needs, develop a case plan with the goal of reducing blood lead levels, educate family regarding lead risk behaviors, schedule blood lead level retest, and refer to appropriate resources regarding lead screening guidelines. An environmental investigation is initiated at this time or may be requested through a physician's order. Environmentalists perform an environmental investigation on a residence to identify lead hazards and recommend interim control or abatement measures if necessary.

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Blood Lead Screening and Management Guidelines

Screening Guidelines

All children should receive blood lead level (BLL) screenings at 12 and 24 months of age. Providers have the option obtaining the lead level and Hct or Hgb at 12 months of age.
A BLL screening is also required for a child who:

- Is 24 to 72 months of age and has not previously received a BLL screening.
- Has a change in risk status.
- Presents with symptoms of possible lead poisoning. (Examples: severe anemia, seizures, constipation, abdominal pain, changes in behavior.)

Lead Risk Assessment Questionnaire

Providers should assess a child's risk of blood lead poisoning beginning at 9 month of age. Children determined to be at high risk of blood lead poisoning should receive parental education, nutritional counseling, and a BLL screening as appropriate. Administering the Risk Assessment Questionnaire instead of a BLL screening does not meet Medicaid requirements. For initial lead screenings a venous specimen is preferred, although capillary samples are acceptable. However, when confirming a BLL, the only acceptable specimen is a venous specimen. Please note the higher the BLL is on the initial screening capillary test, the more urgent it is to get a venous sample for confirmatory testing.

BLL (ug/dL)	COMMENTS
3.5-9	CONFIRM with venous sample within **3 months**
10-19	CONFIRM with venous sample within **1 months**
20-44	CONFIRM with venous sample within **2 weeks**
≥45	CONFIRM with venous sample within **48 hours**

Venous Samples - Confirmed Diagnostic Comments

< 3.5	• EDUCATE families about preventing lead exposure • During well-child visits, check development to make sure age-appropriate milestones are being met and discuss diet and nutrition with a focus on iron and calcium intake. • SCREEN BLL at 12 and 24 months of age, or as indicated by risk status.
3.5-19	• OBTAIN confirmatory diagnostic (venous) test within 3 months, even if the initial sample was venous. • CONTINUE follow-up testing as follows: >3.5-9 in 3 months and 10-19 in 1 month. • EDUCATE families concerning lead absorption and sources of lead exposure (ADPH pamphlet available). Case management services • EXPLAIN that there is no safe level of lead in the blood. • PROVIDE nutritional counseling. • COMPLETE history and physical exam. • TEST for anemia and iron deficiency. • PROVIDE neurodevelopmental monitoring. • SCREEN all siblings under age 6. • OBTAIN abdominal X-ray (if particulate lead ingestion is suspected) with bowel decontamination if indicated.
20-44	• Follow recommendations for BLL >3.5-19 ug/dL. • REFER for targeted case management and environmental investigation via mailing ADPH-FHS-135, *Environmental Surveillance Form,* to the address on the bottom of the form within 3 days of notification of results. • Perform a complete history and physical exam, assessing the child for signs and symptoms related to lead exposure. • Consider performing an abdominal X-ray to check for lead-based paint chips or other radiopaque foreign bodies. This is important for young children who tend to swallow or eat non-food items. Child may also put their mouths on surfaces that could be covered with lead dust. Initiate bowel decontamination if indicated. • PROVIDE parental education and nutritional counseling. • RETEST within 2 weeks – 1 month with venous sample or more often as determined by physician.

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≥45	• Follow recommendations for BLL 20-44 ug/dL. • If the patient exhibits signs or symptoms of lead poisoning, including confusion, weakness, seizures, coma, nausea, vomiting and abdominal pain, admit them to a hospital as soon as possible. • Consider admitting the patient to a hospital if one of these conditions exists: The patient's home is not lead-safe, and they are unable to find a lead-free living space, or the source of lead exposure had not been identified, and the potential for further lead exposure is still possible. • The healthcare provider is consulting with a medical toxicologist or pediatrician with experience in treating lead poisoning to initiate gastrointestinal decontamination or chelation therapy. • REFER for targeted case management and environmental investigation via faxing ADPH-FHS-135, *Environmental Surveillance Form,* to 334-206-2983 immediately upon notification of results. • PROVIDE parental education and nutritional counseling. • RETEST as soon as possible.

***Child should only return to a lead-safe environment after chelation therapy.**

CLINICAL NOTE:

Most children with lead poisoning are asymptomatic. Symptomatic children with elevated blood lead levels should be evaluated immediately. Symptoms may include coma, seizures, bizarre behavior, ataxia, apathy, vomiting alteration of consciousness, and subtle loss of recently acquired skills. Lead encephalopathy has been reported with levels as low as 70 µg/dL.

Environmental Lead

Environmental Lead Investigations is the investigation of the home or primary residence of an EPSDT-eligible child who has an elevated blood lead level. Please refer to Chapter 101, County Health Departments, for more information.

Normal and Abnormal Diagnoses

An abnormal diagnosis should only be billed when a health problem is identified and is referred for further diagnosis and treatment services. These services may be self-referrals.

A normal diagnosis should be billed when no health problem is identified or when identified health problems are treated immediately (acute or one time problem) during the screening (same day) and no referral is made for further diagnosis and treatment services. A normal diagnosis should also be billed when the only referrals are for *routine* vision, hearing or dental services. ICD 10 diagnosis Z00.12, Z76.1 and Z76.2 for age 29 days to 17 years and Z00.0, Z00.00 and Z00.01 for age 18+ on or after October 01, 2015.

Interperiodic Screenings

EPSDT-eligible children may receive medical, vision, hearing, and dental services that are medically necessary to determine the existence of a suspected physical or mental illness or condition, regardless of whether such services coincide with the periodicity schedule for these services. Screenings that are performed more frequently or at different intervals than the established periodicity schedules are called **interperiodic screenings**. An interperiodic screening may be performed before, between, or after a periodic screening if medically necessary. Interperiodic screenings are performed for undiagnosed medically necessary conditions outside the established periodicity schedule. Interperiodic EPSDT screenings are problem-focused and abnormal.

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Interperiodic screening examinations may also occur even in the case of children whose physical, mental, or developmental illnesses or conditions have already been diagnosed. If there are indications that the illness or condition may have become more severe or has changed sufficiently, then further examination is medically necessary.

By performing an interperiodic screening and issuing an EPSDT referral form, physician office and other benefits will be “saved” for acute illnesses or other sickness. An interperiodic screening should be performed (where a history and problem-focused physical exam occurs) for suspected medical, vision, hearing, psychological, or dental problems in order for an EPSDT referral to be issued for further diagnosis and/or treatment. In this manner, the recipient will be referred for consultation and/or to a specialist for medically necessary and appropriate diagnostic tests and/or treatment. Vision/hearing screenings are to be performed/billed on the same date of service as an initial or periodic screening only. Vision/hearing screenings are limited to one each annually, beginning at age 3 for vision and 5 for hearing. However if a suspected vision/hearing/ dental/medical problem should manifest itself, an interperiodic screening should be performed in order for an EPSDT referral to be issued to a specialist or consultant. For more information regarding vision and hearing screenings, please refer to section A.3.5. For more information regarding dental screening and EPSDT referral requirements, please refer to Chapter 13 Dentist.

An interperiodic screening may be performed based upon a request by the parent(s) or guardian(s), or based on the provider's professional judgment relative to medical necessity. The Alabama Medicaid Agency considers **any** encounter with a health care professional who meets the qualifications for participation in the EPSDT program to be an interperiodic screen, regardless of whether the health care professional is enrolled as a provider with the Agency.

A health developmental or educational professional who comes in contact with the child outside the formal health care system may also determine whether an interperiodic screening is medically necessary. The screening provider must document the person referring the child, and a description of the suspected problem, in the record.

Documentation requirements for interperiodic screenings are:

- Consent
- Medical-surgical history update
- Problem-focused physical examination
- Anticipatory guidance/counseling related to the diagnosis made.

Interperiodic screenings must always be filed with the patient’s other insurance first. If the primary insurance is a HMO or the provider is a FQHC, IRHC or PBRHC, the interperiodic screening code must be submitted. Once the claim has been paid/denied, Medicaid may then be billed utilizing the interperiodic screening code with an EP modifier appended. When filing for an interperiodic screening, always append an EP modifier or the visit will count against benefit limits.

If the primary insurance is not a HMO, bill the appropriate “office visit” code. Once the claim has been paid/denied from the patient’s other insurance, a claim may be filed with Medicaid utilizing the same “office visit” code with an EP modifier appended. When billing an office visit code for an interperiodic code, always append the EP modifier or the visit will count against benefit limits.

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NOTE:

If any other treatments are provided the same day (injections, lab, etc.), a “1” or “4” must also be reflected in Block 24h, on each line item, or the claim will deny.

NOTE:

The codes for interperiodic screenings must be billed with an EP modifier and are as follows:

- 99202 EP through 99215 EP for office and/or outpatient interperiodic screenings
- 99233 EP for Inpatient interperiodic screenings

The new interperiodic screening codes will count against office / hospital visit limits if billed without an EP modifier.
The Evaluation and Management code level of care chosen must be supported by medical record documentation.
Each child’s primary insurance must be billed first, and then Medicaid as the payor of last resort.

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SCREENING CODES

PROCEDURE CODE	DESCRIPTION	Medicaid EPSDT Provider Rate (EP)
99381 EP EPSDT NEW PATIENT	NEW PATIENT UNDER 1 YEAR OF AGE	$ 80.00
99382 EP EPSDT NEW PATIENT	NEW PATIENT 1 YEAR TO 4 YEARS OF AGE	$ 80.00
99383 EP EPSDT NEW PATIENT	NEW PATIENT 5 YEARS TO 11 YEARS OF AGE	$ 80.00
99384 EP EPSDT NEW PATIENT	NEW PATIENT 12 YEARS TO 17 YEARS OF AGE	$ 80.00
99385 EP EPSDT NEW PATIENT	NEW PATIENT 18 YEARS TO 20 YEARS OF AGE	$ 80.00
99391 EP EPSDT ESTABLISHED PATIENT	ESTABLISHED PATIENT UNDER 1 YEAR	$ 80.00
99392 EP EPSDT ESTABLISHED PATIENT	ESTABLISHED PATIENT 1 YEAR TO 4 YEARS OF AGE	$ 80.00
99393 EP EPSDT ESTABLISHED PATIENT	ESTABLISHED PATIENT 5 YEARS TO 11 YEARS OF AGE	$ 80.00
99394 EP EPSDT ESTABLISHED PATIENT	ESTABLISHED PATIENT 12 YEARS TO 17 YEARS OF AGE	$ 80.00
99395 EP EPSDT ESTABLISHED PATIENT	ESTABLISHED PATIENT 18 YEARS TO 20 YEARS OF AGE	$ 80.00
99202 EP INTERPERIODIC	NEW PATIENT OFFICE/OUTPATIENT VISIT, TOTAL TIME 15-29 MINUTES	$ 64.61
99203 EP INTERPERIODIC	NEW PATIENT OFFICE/OUTPATIENT VISIT, TOTAL TIME 30-44 MINUTES	$ 91.85
99204 EP INTERPERIODIC	NEW PATIENT OFFICE/OUTPATIENT VISIT, TOTAL TIME 45-59 MINUTES	$ 141.16
99205 EP INTERPERIODIC	NEW PATIENT OFFICE/OUTPATIENT VISIT, TOTAL TIME 60-74 MINUTES	$ 178 .70
99211 EP INTERPERIODIC	ESTABLISHED PATIENT OFFICE/OUTPATIENT VISIT	$ 17.00
99212 EP INTERPERIODIC	ESTABLISHED PATIENT OFFICE/OUTPATIENT VISIT, TOTAL TIME 10-19 MINUTES	$ 31.00
99213 EP INTERPERIODIC	ESTABLISHED PATIENT OFFICE/OUTPATIENT VISIT, TOTAL TIME 20-29 MINUTES	$ 63.95
99214 EP INTERPERIODIC	ESTABLISHED PATIENT OFFICE/OUTPATIENT VISIT, TOTAL TIME 30-39 MINUTES	$ 93.11
99215 EP INTERPERIODIC	ESTABLISHED PATIENT OFFICE/OUTPATIENT VISIT, TOTAL TIME 40-54 MINUTES	$ 125.40

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NOTE:

Effective 1/1/2011, periodic screening codes 99382 EP- 99385 EP and 99392 EP-99395 EP may not be billed in a hospital setting (inpatient or outpatient facility settings).

Intensive Developmental Diagnostic Assessment

An EPSDT Intensive Developmental Diagnostic Assessment is a multidisciplinary comprehensive screening limited to infants' age zero to under two years, and is also limited to two per recipient per lifetime. These screenings are in addition to the routine periodic screenings and must be performed by a qualified EPSDT Intensive Developmental Diagnostic Assessment Screening provider, as approved and enrolled by Medicaid.

NOTE:

Medical necessity is subject to retrospective review by the Alabama Medicaid Agency. Please refer to Chapter 7, Understanding Your Rights and Responsibilities as a Provider, for more information.

Interagency Coordination

The State of Alabama, in conjunction with the Interagency Coordinating Council and the Alabama Department of Rehabilitation Services will implement a system of services to the eligible population (20 USC Section 1471 et seq, Part H), with the assistance of agencies, programs, providers, and the families of eligible infants and toddlers with special needs.

The Alabama Medicaid Agency is one of nine state agencies that hold positions on the Interagency Coordinating Council. The Early Intervention Law legislates a statewide system of early intervention services for eligible infants and toddlers that is comprehensive and coordinated among all disciplines and providers involved and encourages the development of a system of service delivery that includes parents' participation and input. Services that provide early intervention are to be coordinated across agency and provider lines.

The definition of a child eligible for early intervention services includes infants and toddlers under age three who are either: (1) experiencing developmental delay equal to or greater than 25 percent as measure by appropriate diagnostic instruments and procedures in one or more of the following areas: cognitive development, physical development (including vision and hearing), communication development, social or emotional development, adaptive development; or (2) have a diagnosed physical or mental condition which has a high probability of resulting in developmental delay. Early intervention services can include the following:

Audiology	Service coordination
Family training/counseling & home visits	Occupational therapy
Health	Nursing
Medical services for diagnostic/evaluation	Vision services
Nutrition	Physical therapy
Psychological services	Social work
Special instruction	Speech/language pathology
Assistive technology devices & services	Transportation

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The Early Intervention Service Coordinator who receives the Child Find referral will contact the EPSDT provider to obtain the EPSDT screening information and any other pertinent information. In order to coordinate services, once a well child visit (EPSDT) has been completed and a developmental delay has been indicated, contact Child Find, (**800) 543-3098**. Please refer to the Early Intervention Child Find Referral Form at the end of this Appendix or visit Medicaid's website at: www.medicaid.alabama.gov. Additionally, please refer to https://www.rehab.alabama.gov/ for other rehabilitation services.

NOTE:

You may refer a family to Alabama's Early Intervention System (AEIS) in addition to referring the child and family to other appropriate services. AEIS staff is located in seven districts in the state. Please call the toll free number if you are interested in information about local EI resources.

Recommended Health Education Counseling Topics

2 weeks-3 months Nutrition - Spitting up Hiccoughs Sneezing, etc. Safety Need for affection Immunizations Skin and scalp care Bathing frequency How to use a thermometer When to call the doctor	**4-6 months** Nutrition Safety Teething and drooling/dental hygiene Fear of strangers Lead poisoning Immunizations
7-12 months Nutrition Immunizations Safety Dental hygiene Night crying Separation anxiety Need for affection Discipline Lead poisoning	**13-18 months** Nutrition Safety Immunizations Dental hygiene Temper tantrums Obedience Speech development Lead poisoning
19-24 months Nutrition Safety Need for peer relationship Sharing Toilet training Dental hygiene Need for attention and patience Lead poisoning	**3-5 years** Nutrition Safety Dental hygiene Assertion of independence Type of shoes Need for attention Manners Lead poisoning

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6-13 years	14-21 years
Nutrition	Nutrition
Safety	Dental
Dental care	Safety (automobile)
School readiness	Understanding body anatomy
Onset of sexual awareness	Male/female relationships
Peer relationship (male and female)	Contraceptive information
Preputertal body changes	Obedience and discipline
Substance abuse	Parent-child relationships
Tobacco Cessation	Alcohol, drugs, and smoking
Contraceptive information (if sexually active)	Tobacco Cessation
	Occupational guidance
	Substance abuse

Providers may use the PT+3 teaching method for anticipatory guidance counseling. Providers should document PT+3 counseling was utilized and list the three points emphasized.

Providers must provide age-appropriate health education related to smoking and smoking cessation. This includes risk-reduction counseling with regard to use during routine well-child visits. In addition to routine visits, additional counseling must be provided when medically necessary for individuals under age 21.

Billing Requirements

The table below provides billing information for EPSDT screening claims:

Topic	***Explanation***
Copayment	EPSDT recipients, under 18 years of age, are not subject to co-payments.
Prior Authorization	Screenings are not subject to prior authorization.
Referral	Please refer to Section A.4, Providing and Obtaining Referrals, for more information.
Time Limit for Filing Claims	One year from the date of service
Visit Limitations	Effective 05/08/2020 an office visit is now billable on the same day with an EPSDT screening by the same provider or provider group.
Diagnosis Codes	The *International Classification of Diseases - 10th Revision - Clinical Modification* (ICD-10-CM) manual lists required diagnosis codes. These manuals may be obtained by contacting the American Medical Association, AMA Plaza 330 North Wabash Ave, Suite 39300 Chicago, IL 60611-5885, or 1-800-621-8335. Physicians, nurse practitioners, physician assistants, and EPSDT screening providers are able to submit up to 12 additional diagnosis codes per date of service by submitting an additional claim with CPT 99499 (unlisted/unspecified) as a claim detail. Claims for this code should not include other procedure codes. Medicaid will pay $0.00 on claims for CPT 99499. Federally qualified health centers (FQHCs) and rural health clinics (RHCs) should bill CPT 99499 as a fee-for-service claim. Prospective payment rates will not be reimbursed for claims for this code.

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Topic	*Explanation*
Procedure Codes and Modifiers	The following procedure codes should be used when billing comprehensive EPSDT screening services: • 99381;EP - 99385;EP: Initial EPSDT Screening • 99391;EP - 99395;EP: Periodic EPSDT Screening • 99173;EP: Vision Screening – Annual • 92551;EP: Hearing Screening – Annual Effective January 1, 2007, the interperiodic screening codes have changed. The following procedure codes (in service locations other than inpatient hospital) must be used: 99211;EP - 99215;EP EP modifiers are used when billing initial, periodic and interperiodic screenings. Once chronic problems have been identified, providers should bill the appropriate level of office visit code and bill the services as EPSDT referring. For interperiodic screenings performed in an inpatient hospital setting, the following procedure code must be used: 99231;EP – 99233;EP Interperiodic screening codes should also have abnormal diagnosis codes. The (837) Professional, Institutional and Dental electronic claims and the paper claim have been modified to accept up to four Procedure Code Modifiers.
Intensive Developmental Diagnostic Assessment (Multidisciplinary team) An HT modifier must be appended to procedure codes 96110, 96112 and 96113 to identify Intensive Diagnostic Assessment (Multidisciplinary team)	The following procedure codes should be used when billing for an intensive development diagnostic assessment (a multidisciplinary comprehensive screening) for children under two years of age (limited to two per recipient per lifetime) 96110;HT – Developmental testing; limited (eg, Developmental Screening Test II, Early Language Milestone Screen), with interpretation and report. (Formerly known as Intensive developmental diagnostic assessment, normal findings) 96112;HT – First hour 96113;HT – each additional 30 minutes Developmental testing; extended (includes assessment of motor, language, social, adaptive and/or cognitive functioning by standardized developmental instruments) with interpretation and report. (Formerly known as Intensive developmental diagnostic assessment, abnormal findings)
Third Party Coverage	Providers are required to file with available third party resources prior to filing Medicaid. Preventive pediatric services and prenatal care are excluded from this requirement unless the recipient has managed care coverage or Medicaid pays the provider a global fee.

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Topic	*Explanation*
Reimbursement	Governmental screening providers (including physicians) will be paid on a negotiated rate basis, which will not exceed their actual costs. Non-governmental screening providers will be paid their usual and customary charge, which is not to exceed the maximum allowable rate established by Medicaid.
EPSDT Indicator Reference	The EPSDT Indicator will be either a "Y" or "N", as applicable, when using electronic claims only. Providers should bill the appropriate level visit code and bill the services as EPSDT referring for chronic problems that have been identified. This EPSDT Indicator will allow care to be rendered and bypass the 14 or 16 visit limits.

NOTE:

Well child visits (Initial, Periodic, and Interperiodic screenings) do not count against recipient's benefit limits of 14 physician office visits per calendar year when billed with the EP modifier. There is no co-pay for recipients under 18 years of age.

A.3.7 ACHN, Primary Care Case Management (PCCM) Referral Services

Please refer to Chapter 40 for more information regarding the ACHN program.

A.3.8 Billing for ACHN Referred Services

Please refer to Chapter 40 for more information regarding the ACHN program.

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A.4 Providing and Obtaining Referrals

One of the primary purposes of the EPSDT services is to ensure that health problems are diagnosed and treated early before they become more complex and their treatment more costly. A Medicaid eligible child who has received an EPSDT screening may receive additional medically necessary health care. These services are considered above the normal benefit limitations and require a referral from an EPSDT screening provider. Some of these referred services require prior authorization from the Alabama Medicaid Agency. For more information regarding prior authorization, please refer to Chapter 4: Obtaining Prior Authorization. For more information regarding a specific service please refer to the corresponding chapter. The Alabama Medicaid Referral Form (Form 362) must be appropriately completed by the screening physician including the screening date that the problem was identified and the reason for referral.

Providers are not required to complete written referrals to the other providers in the same group. The medical record documentation shall clearly indicate that the PCP did a screening, identified the problem, and an appropriate referral was made.

Referrals to specialists and other physicians outside of the group are required to have a written referral.

A cascading referral is used in situations where more than one consultant may be needed to provide treatment for an identified condition(s). When this situation arises, the original referral form is generated by the EPSDT screening provider. If the first consultant determines a recipient should be referred to another consultant/specialist, it is the first consultant's responsibility to provide a copy of the referral form to the second consultant. This process is continued until the condition(s) have been rectified or in remission, or referral expires, at which time a new screening and referral must be obtained. A new approval/EPSDT screening must be provided anytime the diagnosis, plan of care (care plan, plan of treatment, treatment plan, etc.), or treatment changes. The consultant must contact the PCP for a new referral/screening at that time.

Medical documentation must be present in the recipient's medical record identifying the provider making referrals between consultants. Medical documentation created as a result of a referral between consultants must be associated with the original referral.

If a child is admitted to the hospital as a result of an EPSDT screening, the days will not count against the yearly benefit limit. Facility fees for outpatient visits will not count against the yearly benefit limit if the visit is the result of an EPSDT screening and referral. Services rendered by speech and occupational therapists are covered **only** as the result of an EPSDT screening.

Signature Requirement for Referrals: Effective May 16, 2012:

For hard copy referrals, the printed, typed, or stamped name of the primary care physician with an original signature of the physician or designee (physician or non-physician practitioner) is required. **Stamped or copied signatures will not be accepted**. For electronic referrals, provider certification is made via standardized electronic signature protocol.

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A.4.1 Vision, Hearing, and Dental Referrals

If the EPSDT screening provider chooses to refer a recipient for vision, hearing, and/or dental services, the recipient must be referred to the appropriate provider for diagnosis and/or treatment. After the recipient's vision, hearing, and/or dental service is initiated, the consultant's portion of the EPSDT referral form must be completed by the consultant and the appropriate copy must be returned to the screening provider. Referral forms should be returned in 30 days, from the date of the appointment, or (if no appointment was made) from the date of the screening examination.

NOTE:

If the recipient is one year of age or older and is not under the care of a dentist, the recipient must be referred to a dentist for diagnosis and/or treatment. Follow-up on dental referrals is not required.

A referral form is completed by the screening provider when an abnormality or condition is noted during the child's screening that requires further diagnosis and/or treatment. The referring provider must document the condition(s) within the medical record (either in the medical history or physical exam portion). Medicaid has the right to recoup the screening service fees from the referring provider when a referral is made for a condition not documented in the medical record (in medical history or physical exam portion).

A.4.2 Referrals Resulting from a Diagnosis

If, as a result of a medical, vision, hearing, or dental screening, it is suspected or confirmed that the child has a physical or mental problem, the screening provider must refer the child without delay for further evaluation of the child's health status. Follow-up is required to assure that the child receives a complete diagnostic evaluation. Diagnostic services may include but are not limited to physical examination, developmental assessments, psychological and mental health evaluation, laboratory tests and any x-rays. Diagnosis may be provided at the same time, or it may be provided at a second appointment. For services such as physical therapy, speech therapy, and occupational therapy that require physicians' orders, all orders must be written according to practice guidelines and state/federal law and must include the date and signature of the provider, the service(s) ordered, and the recipient's name. The EPSDT referral form may be considered the physician's order as long as these guidelines are met. The physician's order/prescription date is considered "today's date" on the referral form.

The time limit for completing the referral form (Form 362) requires the form to be completed within 364 days of the date of the screening. If an abnormality or condition is noted during an EPSDT screening and an EPSDT referral form is not issued at the time (for example, sickle cell remission), an EPDST referral may be issued at a later date for the same diagnosis only (for example, sickle cell remission changes to sickle cell crisis). In this instance, the date utilized on the referral form will be the same as the date of the EPSDT screening where the abnormality/condition was noted. If another abnormality or condition occurs that was not diagnosed during an EPSDT screening, or if a condition has changed sufficiently so that further examination is medically necessary, an interperiodic screening should be performed (or periodic screening if it is due) to identify the problem.

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EPSDT referrals are valid for **one year from the date of the EPSDT screening**. Therefore, the maximum time an EPSDT referral remains valid is 12 months from the date of the EPSDT screening. The EPSDT screening date must be current to be valid. The EPSDT screening date may not be backdated or future dated. The date of the EPSDT screening should be documented under “Type of Referral” on form 362, the Alabama Medicaid Agency Referral Form. The EPSDT screening date documented on the Referral Form is the date used to determine the length of time an **EPSDT referral is valid**. The “Length of Referral” is used to determine the amount of time the referral is valid from the referral date and is inclusive of all types of referrals (EPSDT referral, Targeted Case Management, etc). Please refer to Appendix E, Medicaid Forms, for additional information.

Diagnosis and treatment services may be provided by the screening provider (self-referral) or may be obtained by referral to any other practitioner or facility qualified to evaluate, diagnose, or treat the child's health problem.

NOTE:

The number of visits or months must be documented on the EPSDT referral form to be considered a valid referral.

A.4.3 Treatment

Treatment may include but is not limited to physicians' or dentists' services, optometrists' services, podiatrists' services, hospital services (inpatient and outpatient), clinic services, laboratory and X-ray services, prescribed drugs, eyeglasses, hearing aids, prostheses, physical therapy, rehabilitation services, psychological services, and other types of health care and mental health services.

If a condition requires a referral, it is the responsibility of the screening provider and PCP, if applicable, to:

- Document the abnormality discovered during the EPSDT screening in the record
- Determine what resources a child needs and to which provider he/she wishes to be referred (the recipient's freedom of choice of providers must be ensured)
- Make the appropriate referral in a timely manner
- Offer and provide assistance in scheduling the appointment
- Verify whether the child received the service. Referrals must be followed up within 30 days (excluding dental) from the date of the appointment with the consultant.

A.4.4 Completing the Referral Form

The Referral for Services Form 362 must be completed after a screening if further diagnosis and/or treatment are required for a child not assigned to a PCP. The referral form is completed when referring the recipient to other providers for services that were identified during the screening as medically necessary.

Refer to Appendix E, Medicaid Forms, for a sample of the Alabama Medicaid Agency Referral Form.

Screening providers must include their 10-digit National Provider Identifier (NPI), name, and address for all recipients.

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- The **screening provider** must document the time span in which the referral is valid. The maximum time span is 12 months from the date of the screening.
- The **consulting provider** must follow the appropriate billing instructions and guidelines for completion of the CMS 1500 claim form found in Chapter 5, Section 5.2.2 of the Alabama Medicaid Provider Manual.

NOTE:

Providers should write the referral as soon as the condition is noted so that the regular benefits are not exhausted. This is important for patients with chronic conditions or a problem that will require numerous visits to treat.

EP modifiers are used when billing initial, periodic and interperiodic screenings. Once chronic problems have been identified, providers should bill the appropriate level of office visit code and bill the services as EPSDT referring.

Once benefit limitations have been exceeded, Medicaid will not pay for services without the EPSDT referral.

The referral form should follow the recipient for all services related to the condition noted on the form. If a child is screened with a particular condition noted and referred for further diagnosis, and another condition develops that is not noted on the referral form, the child must be re-screened in order to receive expanded benefits for the second condition noted. If not re-screened, the services rendered would count against the child's routine benefit limits.

NOTE:

If the screening provider refers a child to a consultant, it is the screening provider’s responsibility to follow up. However, if the managed care provider refers the child to a consultant, it is the managed care provider’s responsibility to follow up.

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A.4.5 EPSDT Referrals for ACHN Recipients

Scenario: A child is referred by the PCP to be screened by a county health department and appears to have a foot deformity.

Procedure: The child **must** be seen by a screening provider (a pediatrician credentialed as a screening provider: for example) to obtain an EPSDT referral form. The PCP may choose to

- Provide the necessary treatment
- Refer the child to an orthopedic specialist
- Instruct the screening provider to complete the referral form

The screening provider must complete the Alabama Medicaid Agency Referral Form (Form 362) if referring the child to a specialist. The referring number reflects the NPI of the screening provider.

The name and address of the screening provider should be entered to reflect, in this scenario, the county health department. All services in this scenario are covered by the original EPSDT screening referral, which must follow the child from visit to visit. The screening NPI and signature will reflect the county health department number and the signature of the health department employee who performed the screening.

The consulting provider must use the screening provider number as the referring physician on the claim form. Each provider testing the condition diagnosed during the screening, and documented in the referral, must include the screening/referring provider's number on the claim form. Please refer to Chapter 5, Filing Claims, for instructions on including the referring NPI on the claim form.

In this scenario, the specialist may suggest surgery, braces, and/or therapy. All services approved by and referred by the screening provider would then be covered by an EPSDT screening referral.

NOTE:

The PCP must be contacted and approve any and all referrals made by the specialist.

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A.4.6 Billing Instructions for Referred Services

For EPSDT Referred Services

If you file hard copy claims on the **UB-04**, you must complete the following fields:

- Block 2 – Enter the screening provider's 10-digit National Provider Identifier (NPI)
- Block 24 – Enter "**A1**" to indicate EPSDT

If you file **electronically** on the UB-04 (837 Institutional) using Gainwell *Provider Electronic Solutions* software, please refer to the latest edition of the Provider Electronic Solutions User Manual.

If you file claims on the **CMS-1500**, you must complete:

- Block 17 – Enter the name of screening provider
- Block 17a – Enter the screening provider's 10-digit National Provider Identifier (NPI)
- Block 24H – Enter "**1**" to indicate EPSDT

If you file **electronically** on the CMS-1500 (837 Professional) using Gainwell *Provider Electronic Solutions* software, please refer to the latest edition of the Provider Electronic Solutions User Manual.

For EPSDT Referred Services

If you file claims on the **UB-04**, you must complete:

- Block 2 – Enter the referring PCP's 10-digit National Provider Identifier (NPI)
- Block 24 – Enter "**A1**" to indicate EPSDT and managed care

If you file electronically on the UB-04 (837 Institutional) using Gainwell *Provider Electronic Solutions* software, please refer to the latest edition of the Provider Electronic Solutions User Manual.

If you file claims on the **CMS-1500**, you must complete:

- Block 17 – Enter the name of referring PCP
- Block 17a – Enter the referring PCP's 10-digit National Provider Identifier (NPI)
- Block 24H – Enter "**4**" to indicate EPSDT and managed care

If you file **electronically** on the CMS-1500 (837 Professional) using Gainwell *Provider Electronic Solutions* software, please refer to the latest edition of the Provider Electronic Solutions User Manual.

NOTE:

Each line item on the CMS-1500 Claim Form must have an indicator in Block 24 or 24H if billing for a Referred Service.

For Example: If the first line is an office visit and the indicator in Block 24 Or 24H is a "4", All additional services for that Date Of Service must also have an indicator of "4" in Block 24 Or 24H or the claim will deny.

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Coordinating Care

The Alabama Medicaid Agency establishes the service standards and requirements that the providers must meet.

Providers of medical screening services are responsible for overall care coordination for those recipients that are not enrolled in a managed care system. For those recipients who are enrolled in the ACHN, it is the ACHN's responsibility for overall care coordination. These ongoing activities include scheduling, coordinating, follow-up, and monitoring necessary EPSDT screening and other health services.

Care coordination enhances EPSDT Program efficiency and effectiveness by assuring that needed services are provided in a timely and efficient manner and that duplicated and unnecessary services are avoided.

A.4.7 Consent Forms

Since EPSDT screenings are voluntary services, some parents of children may decline a screening. This does not preclude the child from receiving a screening at a later date or receiving medically necessary diagnosis, treatment or other health services separate from the screening, providing such services do not exceed normal benefit limitations.

A "Consent for Services" form must be signed at each visit by the responsible adult. The consent could be a permission form to treat or a signature reflecting the date the service is rendered (e.g., a sign-in sheet). The consent for services should be filed in the patient's permanent medical record. If a sign-in logbook is used, the provider will need to keep this record for a minimum of three years plus the current year. The responsible adult must be present at the time of the screening to give pertinent history and developmental status and to receive counseling as indicated. The absence of a responsible adult as defined above would invalidate the screening. When off-site screenings are performed, the parent may complete the history form prior to the screening in compliance with Off-Site Screening Protocol. Recipients 16 years of age or older may sign for themselves.

A.4.8 Medical Records

All screening providers must maintain complete records for three years plus the current year on all children who have received services or screened. Records of all EPSDT-eligible children must be made available to Medicaid upon request. Medicaid will monitor EPSDT services provided by screening physicians or agencies on a periodic basis. If Medicaid identifies claims paid where any three findings listed as critical components of the screening process are omitted, the claim may be adjusted.

Medical records must include the following documentation. The critical components of a well child visit (comprehensive screening) are denoted with an asterisk.

- Consent signature
- * Family history of diseases and annual updates
- * Medical history and updates at each screening
- Mental health assessment
- * History of immunizations and administration as indicated
- * Age-appropriate developmental assessment

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- * Age-appropriate anticipatory guidance
- * Nutritional assessment to include recorded results of hemoglobin/hematocrit and plotted height/weight
- * Documentation of sickle cell test results
- * Recorded results of hemoglobin/hematocrit
- *Urine screening needs to be performed only when clinically indicated and an appropriate medical diagnosis is required.
- * Lead testing/results (according to age)
- Tb skin test
- Height, weight, temperature, pulse, and blood pressure
- * Vision and hearing assessment/testing (Considered as two critical components)
- * Documentation of the unclothed physical examination
- * Dental referral/status for recipients 1 year of age and above
- * Failure to make appropriate referral, when required (i.e., medical, vision, hearing)
- * Referral follow-up on conditions related to medical, vision, or hearing problems

A.5 Justice Involved Youth (JIY) Travel Payment

A.5.1 Purpose and Policy Overview

Added: Justice Involved Youth…documentation standards, referral

Purpose of the JIY Travel Payment

The Alabama Medicaid Agency (Medicaid) developed the JIY Travel Payment to provide financial reimbursement to EPSDT providers conducting EPSDT screenings on justice involved youth in carceral facilities. The payment provides reimbursement for travel between the provider's office and the carceral facility and additional travel-related time required to deliver services in a non-traditional setting.

The JIY Travel Payment is funded through the Consolidated Appropriations Act (CAA) Section 206 Planning Grant1, a four-year federal funding opportunity through the Centers for Medicare & Medicaid Services (CMS) that helps state Medicaid agencies improve care continuity for Medicaid and CHIP-eligible individuals during and after incarceration. The grant will provide funding for the JIY Travel Payment.

Policy Authority and Relationship to Existing EPSDT Policy

Added: rules, and medical…in this section.

The JIY Travel Payment is an add-on payment to an EPSDT service. Therefore, the EPSDT benefit is the underlying Medicaid authority for the Travel Payment. The JIY Travel Payment policy does not replace or modify any current EPSDT service definitions. All EPSDT screening requirements, documentation standards, referral rules, and medical necessity criteria in Appendix A remain applicable unless explicitly modified in this section.

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A.5.2 Provider Eligibility

Added: Provide Eligibility Eligible…screen ings enrollment process

Eligible Providers

Providers must be enrolled and contracted with Medicaid to be eligible for the JIY Travel Payment. Additionally, providers must have submitted an EPSDT Agreement to Gainwell Provider Enrollment to be eligible for the payment. It is the responsibility of the provider to maintain an active EPSDT Service Agreement.

In accordance with Appendix A, only the following provider types are eligible to be EPSDT Screening Providers and to receive the JIY Travel Payment:

- Physicians
- Nurse Practitioners
- Registered Nurses (see information in Section A.3)
- Physician Assistants

Some providers may have additional eligibility restrictions based on their provider type. For additional information on the Medicaid Enrollment and the EPSDT provider requirements see Chapter 2. Becoming a Medicaid Provider and Section A.3.1 Becoming an EPSDT Screening Provider of the Provider Billing Manual.

Additional JIY Program Requirements

In addition to the requirements above, to be eligible for the JIY Travel Payment, EPSDT providers must be authorized to perform EPSDT screenings in offsite or non-traditional settings consistent with policies and requirements outlined in Appendix A. EPSDT providers must complete an off-site provider enrollment process before providing services or being eligible for the payment. Key steps in the off-site provider enrollment process include:

- Demonstrating that services will be offered to all children enrolled at an off-site location, not just Medicaid-eligible children.
- Developing a signed Matrix of Responsibilities form between the off-site location (i.e., carceral facility) authority and the screening provider. Only one screening provider can be approved per location.
- Maintaining a monthly schedule of dates, times, and facilities where the EPSDT services will be offered.

See section A.6 Off-site Screenings for additional information on the off-site screenings' enrollment process.

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A.5.3 Covered Services and Payment Structure

Added: Covered Services and…provider enrollment process

Covered Encounters

EPSDT providers are eligible to receive the JIY Travel Payment when provider travel is required to provide ESPDT screening services to JIY recipients in carceral settings. The types of EPSDT screenings eligible for the add-on payment include:

- Initial Screenings: The first time an EPSDT screening is performed on a recipient by an EPSDT screening provider.
- Periodic Screenings: Well child visits performed based on a periodicity schedule.
- Interperiodic Screenings: Problem-focused and abnormal screening performed when medically necessary for undiagnosed conditions outside the established periodicity schedule and can occur at any age.

EPSDT providers may screen more than one JIY recipient during a single visit to a carceral facility. Providers may also visit the facility more than once to see the same recipient if additional screening is required.

Payment Methodology

EPSDT providers will receive a flat $50 JIY Travel Payment per eligible EPSDT encounter. The JIY Travel Payment is in addition to the Youth Justice EPSDT screening rate ($88) and is intended to reimburse the provider for travel and time spent traveling to a non-traditional setting. The rate will remain the same regardless of travel-related factors including:

- Distance traveled;
- Time spent traveling

EPSDT providers are limited to one JIY Travel Payment per date of service and two JIY Travel Payments per calendar year per JIY recipient. However, if a provider serves multiple JIY recipients at a single location on the same date, they may claim a JIY Travel Payment per youth served. Reimbursements will be paid in accordance with policies outlined in Provider Billing Manual Section 6. Receiving Reimbursement and JIY Travel Payment limitations do not impact reimbursement for EPSDT screenings.

The JIY Travel Payment will not provide reimbursement beyond the $50 flat fee for travel-related or other costs of conducting the EPSDT screening. Reimbursement does not include:

- Mileage separate from the flat Travel Payment
- Reimbursement for the EPSDT encounter (EPSDT encounters may be reimbursed separately)
- Staff travel costs outside of the EPSDT provider
- Planning and coordination to complete the off-site provider enrollment process

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A.5.4 Billing and Claims Submission

Claims including the JIY Travel Payment must be submitted in accordance with Medicaid billing requirements for EPSDT services and all applicable Provider Billing Manual instructions (See Section A.3.6 Critical Components of Screenings). Payment of the JIY Travel Payment is contingent upon the submission of a payable EPSDT screening claim and compliance with all billing, eligibility, and documentation requirements.

Added: Billing and Claims…to Section A.3.6

Procedure Code Application

Medicaid may reimburse eligible EPSDT providers for travel associated with furnishing EPSDT screenings to JIY program recipients using CPT code 99429. The JIY Travel Payment is reimbursed at a flat rate of $50 dollars per date of service and is payable in addition to the $88 Youth Justice EPSDT screening rate.

The JIY Travel Payment will be paid only when the provider has a paid EPSDT screening in claims history and when the code is billed with a payable EPSDT screening code.

The EPSDT screening code and CPT 99429 may be submitted on the same claim for payment.

Place of Service Requirements

Claims billed with CPT 99429 must include Place of Service code 09 – Prison/Correctional Facility. The Place of Service requirement applies to both the EPSDT screening code and CPT 99429. Payment of the JIY travel payment is contingent upon use of the required Place of Service code. Providers are responsible for ensuring that the Place of Service code used on the claim accurately reflects the location where the EPSDT screening was furnished.

Provider Enrollment and Specialty Requirements

Only providers enrolled with Medicaid and assigned Specialty 560 (EPSDT Provider) are eligible to bill EPSDT screening services and CPT 99429. Providers must maintain active enrollment and specialty designation at the time services are rendered and billed. For additional information on Specialty 560 (EPSDT Provider) requirements please refer to Section A.3.

Additional Billing Requirements

All standard EPSDT billing requirements apply when billing CPT 99429, including third-party liability requirements. However, the JIY Travel Payment is payable even if third-party liability has been established on the underlying EPSDT screening claim.

All EPSDT screening requirements, including the Body Mass Index (BMI) documentation requirement, apply to EPSDT screenings furnished in carceral or correctional settings when Place of Service code 09 is used. For additional information on EPSDT screening requirements please refer to Section A.3.6.

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Billing Limitations

Added: Billing Limitations Payment…or approved placement

Payment for CPT 99429 is limited to two (2) payments per calendar year per JIY recipient.

Certified Registered Nurse Practitioners (CRNPs) and Physician Assistants (PAs) will be reimbursed at eighty percent (80%) of the $50.00 rate for CPT 99429, consistent with Medicaid payment policy.

Federally Qualified Health Centers (FQHCs) must bill CPT 99429 outside of the Prospective Payment System (PPS) rate, and CPT 99429 is exempt from PPS reimbursement.

A.5.5 Documentation Requirements

Providers billing for the JIY Travel Payment must maintain documentation sufficient to support both the underlying EPSDT screening and the application of the JIY Travel Payment. Documentation must be complete, accurate, legible, contemporaneous, and attributable to the date of service. All documentation must be retained and made available to the Alabama Medicaid Agency or its designee upon request.

Documentation requirements for the JIY Travel Payment are additive to and does not replace existing EPSDT documentation standards set forth in Appendix A and other applicable Provider Billing Manual chapters. Claims billed with the JIY Travel Payment are subject to post payment review and audit, and payments may be denied or recouped if documentation is incomplete, inconsistent, or insufficient to support the modifier.

General EPSDT Documentation Requirements

Providers must comply with all EPSDT medical record requirements applicable to the screening service billed. The JIY Travel Payment does not modify EPSDT medical necessity standards, screening content requirements, referral obligations, or follow-up expectations. For additional information on EPSDT medical record requirements, refer to Chapter 7 of the Provider Manual.

Required Justice Involved Youth Documentation

The medical record must include documentation sufficient to demonstrate that the individual receiving services was participating in Alabama's JIY Program on the date the EPSDT screening was furnished. Documentation must clearly establish the context of care delivery as related to the JIY population.

Acceptable documentation may include, but is not limited to:

- Notation within the progress note indicating the youth's participation in a justice related placement, program, or court supervised setting;
- Identification of the service location as a justice related or court authorized site;
- Documentation of coordination with a justice involved program representative, agency, or approved placement.

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Providers should not maintain confidential court records or legal documents in the medical record; however, documentation must be adequate to substantiate eligibility for the JIY Travel Payment upon review.

Added: Providers should not…date of service:

Travel and Time Documentation Requirements

Because the JIY Travel Payment is intended to compensate EPSDT providers for travel and travel related time required to furnish services to justice involved youth, providers must maintain travel documentation tied to each EPSDT screening encounter billed with the travel payment procedure code. At a minimum, documentation must include all of the following for each encounter billed with the JIY Travel Payment procedure code:

- Mileage traveled to and from the service location;
- Departure and return times, including:
  - Time departed the provider's primary work location or designated office;
  - Time arrived at the justice related service location;
  - Time departed the service location following completion of services; and
  - Time returned to the provider's primary work location or office.

Travel documentation must correspond directly to the specific date of service for which the JIY Travel Payment is billed. If a provider serves multiple JIY recipients at a single location on the same date, required documentation must be maintained for each recipient. Mileage or time estimates, pooled travel logs not linked to individual encounters, or documentation created significantly after the date of service may not be sufficient to support payment.

Acceptable Documentation Formats

Medicaid recognizes that providers may use a variety of documentation formats to support the JIY Travel Payment. Documentation may be maintained in electronic or paper form, provided it is complete, auditable, and retrievable upon request. Acceptable documentation formats may include, but are not limited to:

- EHR progress notes that include a discrete travel section or narrative documenting mileage and travel times;
- Separate travel logs or worksheets (if applicable), maintained contemporaneously with the date of service and as part of the patient's medical record, that include the required mileage and time elements and are clearly associated with the EPSDT screening encounter;
- Scanned or electronically stored supporting documents, such as mileage logs or time records, maintained as part of the patient's medical record or an auditable supplement to the record.

Regardless of format, documentation must clearly identify:

- The EPSDT screening encounter billed;
- The date of service;

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Added: The provider rendering…EPSDT Service Agreement

- The provider rendering services; and
- The travel is associated with furnishing services to the Justice Involved Youth.

Documentation maintained outside the core medical record (e.g., centralized travel logs) must be readily cross referenced to the specific EPSDT encounter and made available to Medicaid upon request.

Relationship Between Travel and the EPSDT Screening Encounter

Documentation must demonstrate that reported travel and travel related time were directly attributable to furnishing an eligible EPSDT screening to a recipient in the JIY Program. The JIY Travel Payment may not be billed when:

- Services are provided at the provider's usual place of business without travel;
- Travel does not occur on the date of service; or
- Services furnished do not meet EPSDT screening requirements.

Only one JIY Travel Payment may be billed and supported per eligible EPSDT screening encounter, per youth, per date of service. Documentation must reflect compliance with this limitation.

Record Retention

Providers must retain all documentation supporting payment of the JIY Travel Payment for a minimum of three (3) years plus the current year, consistent with EPSDT record retention requirements and Medicaid policy.

Records must be available for post payment review, audit, or program integrity activities. Documentation inconsistencies, omissions, or failure to produce requested records may result in claim denial or recoupment in accordance with Medicaid policy.

A.5.6 Program Integrity and Oversight

Medicaid will oversee, monitor, and enforce appropriate use of the JIY Travel Payment through existing EPSDT program integrity, utilization monitoring, post payment review, and audit processes. Oversight activities are intended to ensure that JIY Travel Payments are made only for allowable EPSDT screening encounters furnished to eligible JIY Program recipients, and that all billing, documentation, and contractual requirements are satisfied.

Please note the JIY Travel Payment does not establish a separate oversight framework. Claims billed with the JIY Travel Payment procedure code are subject to the same compliance standards, monitoring authority, and enforcement mechanisms applicable to all EPSDT services under Medicaid policy and the EPSDT Service Agreement.

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Post-Payment Review and Monitoring

Claims billed with the JIY Travel Payment procedure code may be subject to post payment review to verify compliance with JIY Travel Payment requirements. Reviews may be conducted on an individual claim basis or as part of broader monitoring activities.

Added: Post-Payment Review…not limited to:

Post payment reviews may include, but are not limited to, desk reviews, focused or targeted audits, utilization reviews, data analytics based monitoring, or other program integrity activities conducted by Medicaid or its designee.

During post payment review, Medicaid may verify, at a minimum:

- That an eligible EPSDT screening service was furnished on the billed date of service;
- That the individual receiving services was eligible for services under the JIY Program on the date the service was furnished;
- That travel and travel related time occurred and were directly attributable to furnishing the EPSDT screening;
- That the JIY Travel Payment was billed in accordance with policy limitations, including the one modifier per encounter restriction; and
- That all documentation requirements set forth in Section A.5.5 were met.

Documentation Review Standards

Providers must produce documentation sufficient to support both the EPSDT screening and the application of the JIY Travel Payment upon request by Medicaid or its designee. Documentation reviews will assess whether records:

- Clearly and consistently support the services billed and qualify for the travel payment;
- Are attributable to the specific date of service and individual encounter;
- Are internally consistent across medical records, travel documentation, billing submissions, and related materials; and
- Demonstrate compliance with all applicable EPSDT and Service Agreement requirements.

Documentation that is incomplete, conflicting, not contemporaneous, or insufficient to substantiate the application of the JIY Travel Payment procedure code may result in denial or recoupment of payments, even when the underlying EPSDT screening service is otherwise payable.

Utilization Monitoring and Billing Controls

Medicaid may conduct ongoing utilization monitoring to identify billing patterns or practices that warrant additional review or corrective action. Monitoring activities may include analysis of claims data, comparison of provider billing patterns, and identification of outlier utilization.

Billing practices that may be subject to further review include, but are not limited to:

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Added: Repeated billing of…payment as well.

- Repeated billing of the JIY Travel Payment without documentation supporting travel;
- Billing of the JIY Travel Payment for services furnished at the provider's usual place of business;
- Billing of more than one JIY Travel Payment per EPSDT screening encounter, per youth, per date of service; or
- Patterns suggestive of duplicate, excessive, or unsupported travel payment use.

Identified concerns may result in provider education, focused audits, corrective action requirements, or other program integrity actions as determined appropriate by Medicaid.

Recoupment

When improper billing or payment of the JIY Travel Payment is identified, Medicaid may pursue recovery of payments in accordance with state and federal recovery requirements. Providers may be required to implement corrective actions to address identified deficiencies, which may include additional training, revisions to documentation or billing practices, or enhanced internal controls.

All corrective actions, recoveries, and enforcement actions will be conducted in accordance with applicable federal and state requirements, Alabama Medicaid program integrity policies, and the EPSDT Service Agreement. If Medicaid recoups payment for an EPSDT screening billed with a JIY Travel Payment, the recoupment will apply to the JIY travel payment as well.

A.6 Off-site Screenings

Children are our state's most important assets and yet many of them arrive at school generally in poor health. The healthier a child, the greater his or her learning potential. The Alabama Medicaid Agency is committed to helping ensure that children are healthy and ready to learn. To that end, the Alabama Medicaid Agency has developed protocols for off-site EPSDT screenings. These services must be accessible to all children, not just Medicaid-eligible children.

NOTE:

EPSDT screening providers must also contact the recipient's primary care physician to receive prior authorizations to perform the screening.

Off-site screenings are defined as screenings that are provided off-site from a medical facility, which is limited to hospitals, physician offices, Department of Public Health (DPH) clinics, and Federal/State certified clinics. Off-site screenings occur in schools, day care centers, head start centers, and housing projects.

An off-site EPSDT screening provider must develop and adhere to confidentiality policies set out by the respective agencies and should be submitted to the agency. Information pertinent to the child's performance may be shared. Information pertinent to infectious disease shall be released only by the County Health Officer. Sharing

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information with others outside the local agency may take place only if parental consent has been given.

Provider is defined as and will include only a county health department clinic, hospital, FQHC, IRHC, PBRHC, or a physician's office. A provider must be located within the county or within 15 miles of the county in which the off-site screenings occur. Medical personnel performing the physical examination are limited to physicians, certified registered nurse practitioners (CRNP), certified nurse midwives (CNM), physician assistants (PA), and registered nurses (RN) employed by the facilities listed above.

Clinic is defined as a certified medical facility, under the supervision of a physician that provides a full range of medical services on a regular basis. A clinic must be equipped to handle acute care situations and provide treatment and/or management of chronic diseases. Licensed medical personnel must perform medical services.

Medical facility is defined as a Federal/State certified clinic, hospital, physician's office, or a DPH clinic where diagnosis of health problems are rendered and treatment of diseases occur. The medical facility must have a permanent location, regularly scheduled hours of operation, and a published telephone number. Medical services and supplies must also be available for treatment of abnormal conditions identified at the time of an EPSDT screening.

Physician's office is defined as a place staffed by physician(s) and other medical professionals where medical activities, such as the practice of medicine, is conducted. This office is specifically designed and set up to provide medical diagnosis and treatment of medical conditions. This office is open and operating on a published, regularly scheduled basis with a published telephone number and regularly scheduled appointments.

A.6.1 Enrollment for Off-site providers

To be considered as an EPSDT screening provider for off-site screenings, potential providers must submit the following criteria:

- A letter documenting the ability to complete all components of a screening. The physical exam portion of the screening must be completed by an approved EPSDT screening provider: physician, nurse practitioner, physician assistant, or a registered nurse. All registered nurses, except BSNs, must complete a Medicaid-approved Pediatric Assessment course or show proof of having completed a similar program of study in their professional training that prepared them to perform pediatric health assessments.
- A primary care referral list of medical providers in the county to whom you will refer to services. The referral list must include pediatricians, family and/or general practice physicians, internal medicine physicians, vision and hearing providers, and dentists. All providers must agree to be on your referral list, therefore, you must submit their written agreement with your referral list. The list must be sufficient in number to allow recipients/parents a choice in the selection of a provider.
- Documentation to demonstrate that services will be offered to all children enrolled at an off-site location, not just Medicaid-eligible children. A copy of your fee schedule must be attached to your documentation and must include fees for non-Medicaid enrollees.
- Child abuse and confidentiality policies

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- A signed Matrix of Responsibilities form between the off-site location authority (school superintendent, principal, day care director, etc.) and the screening provider. Only one screening provider will be approved per location.

NOTE:

Only RNs that are employed by a FQHC, RHC, Health Department, Physicians office, and hospital may perform off-site EPSDT screenings.

- A signed agreement/letter from a local physician to serve as Medical Director. This physician may be a pediatrician, family practice physician, general practice physician, or an internal medicine physician. Proof of 6 pediatric focused credits (CME) from the previous year must be included with the signed agreement. EXCEPTION: A board-certified pediatrician should submit a copy of current certification only. **The medical director is responsible for resolving problems that the nurses encounter and rendering care for medical emergencies.**

- A monthly schedule shall be maintained designating the dates, times, and the local agency in which you will be offering the EPSDT services. The monthly schedule should be readily available and retained in either the local agency/medical facility (i.e., the facility that has been approved as an off-site EPSDT screening provider) or the recipient's medical record. Failure to maintain schedules one week in advance of Off-site EPSDT screenings may result in termination and loss of revenue.
- A document, listing members of the Peer Review Coalition of community members to serve in an advisory capacity. The committee must have the opportunity to participate in policy development and program administration of the provider's off-site program and to advise the director about health and medical service needs within the community. The committee must be comprised of parents, school personnel, public health personnel and local physicians within the local community. Members must be familiar with the medical needs of low-income population groups and with the resources available in the community.
- Information packet materials, including letters, forms, and examples of anticipatory guidance information sheets to be used. These materials must be prior approved by Medicaid.
- A copy of the waiver certificate and/or CLIA number, issued by the Division of Health Care Facility, Bureau of Health Provider Standards for the State of Alabama Department of Public Health.
- A list of all physical locations at which EPSDT screenings will be provided. A separate NPI will be assigned to each off-site location and will be distinct from any other NPI. A separate application and contract is required for each off-site location.

A.6.2 Space for Screenings

The room in which screenings are done may vary according to the availability of space. Space to perform the screening assessment must include a well-lighted private room in close proximity to hot and cold running water, a bathroom, and a nearby waiting area.

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A.6.3 Parent/Guardian Consent and Follow-up

Children under 16 years of age must have written consent from their parent/guardian before participating in the screening program. Children age 16 and above may consent for themselves. The parent/guardian should be encouraged to be present during the screening.

Once the health screening is complete, the parent/guardian must be informed of the results of the screening by mail or in a one-on-one meeting. The anticipatory guidance materials must be age appropriate and the material may be given to children 16 years of age and above. Documentation must reflect that anticipatory guidance materials were mailed to parent/guardian for recipients under 16 years of age.

NOTE:

The potential provider cannot begin well child visits (screenings) until approval has been authorized in writing and Medicaid has enrolled the provider for off-site screenings.

A.7 Vaccines for Children

In an effort to increase the immunization levels of Alabama's children by two years of age, the Alabama Department of Public Health and the Alabama Medicaid Agency implemented the Vaccines for Children (VFC) Program on October 1, 1994,

This nationally sponsored program offers free vaccines to family and general practitioners, pediatricians, hospital nurseries, emergency rooms, and other qualified providers for children aged 18 years and under who are Medicaid enrolled and eligible, have no health insurance, or are American Indian or Alaskan Native. Free vaccines are also available to children who do not have health insurance for immunizations (“underinsured”), if they obtain those vaccines from a Federally Qualified Health Center or Rural Health Clinic.

Participation in Medicaid is not required for VFC enrollment; however, over 400,000 of Alabama’s children are Medicaid enrolled.

A.7.1 Fees

Medicaid has taken the past vaccine and administration fee costs and calculated an equivalent reimbursement fee of $8.00 per dose. When multiple doses are given during the same visit, Medicaid will reimburse for each dose. When doses are given in conjunction with an EPSDT screening or physician visits, an administration fee of $8.00 per injection will also be paid for recipients 18 years of age and younger. The statewide fee-for-service will be paid for recipients 19 years of age and older. Providers are encouraged to utilize licensed available combination vaccines when indicated rather than the individual components of the vaccine.

Providers should use the immunization(s) procedure code designated by the VFC Program when billing for the administration of an immunization. Please refer to section A.6.3 for the list of designated VFC procedure codes.

Medicaid VFC providers may give VFC vaccines to children who are Medicaid enrolled, uninsured, American Indian, or Alaskan Native. If a VFC vaccine is given to any of the above patients, with the exception of Medicaid enrolled, an administration fee not to exceed $19.79 for each vaccine administered may be charged. Underinsured patients must go to an FQHC or RHC, to receive VFC vaccines. . No

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VFC-eligible patient should be denied immunizations because of an inability to pay the administration fee.

Physicians and health departments are not required to file recipient health insurance prior to filing Medicaid for preventive pediatric services, including administration fees for VFC. Exceptions to this rule require that all providers must file with a recipient's health plan when the plan is an HMO or other managed care plan. In addition, FQHCs and RHCs are required to file other insurance prior to filing Medicaid as are any providers receiving a lump sum payment for bundled services or a capitation payment from Medicaid.

A.7.2 Enrollment

The Department of Public Health is the lead agency in administering the VFC Program. Enrollment and vaccine order forms are available through the Immunization Division. Questions regarding enrollment should be directed to the VFC Coordinator at (800) 469-4599.

Participation in Medicaid is not required for VFC enrollment. Participation in the VFC Program is not required for Medicaid enrollment.

A.7.3 Vaccines for Children Billing Instructions

Providers must use an appropriate CPT code on a CMS-1500 claim form or UB-04 claim form in order to receive reimbursement for the administration of each immunization given from VFC stock.

When immunizations are given in conjunction with an EPSDT screening visit or physician office visit, an administration fee of $8.00 per injection will be paid for recipients 18 years or younger. The statewide fee-for-service rate will be paid for recipients 19 and 20 years old.

NOTE:

A VFC provider may or may not choose to become an enrolled Medicaid provider. Enrollment as a VFC provider or a Medicaid provider is independent of each other.

The following CPT codes must be used when billing Medicaid for immunizations for any recipient under age 19:

CPT Procedure Code	*Procedure Description*	*Lay Term Description*	*Brand Name Example(s)*
90380	RSV MAB IM 0.5 ML	RSV Monoclonal Antibody	Beyfortus
90381	RSV MAB IM 1 ML	RSV Monoclonal Antibody	Beyfortus
90382	RSV MONOC ANTB SEASN IM 0.7ML	RSV Monoclonal Antibody	Enfosia
90587	DENGUE VACC QUAD 3 DOSE SUBQ	Dengue vaccine	Dengvaxia
90611	SMALLPOX&MONKEY POX VAC 0.5ML	Smallpox and Monkeypox vaccine	
90619	MENACWY-TT VACCINE IM	Meningococcal conjugate vaccine	MenQuafi
90620	MENB-4C VACC 2 DOSE IM	Menengococcal B vaccine	Bexsero

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CPT Procedure Code	*Procedure Description*	*Lay Term Description*	*Brand Name Example(s)*
90621	MENB-FHBP VACC 2/3 DOSE IM	Meningococcal B vaccine	Trumenba
90623	MENACWY-TT MENB-FHBP	Meningococcal serogroup A, B, C, W, Y vaccine Children aged 10 years or older may receive a dose of Penbraya as an alternative to separate administration of MENACWY and MENB when both vaccines would be given on the same clinic day. For age-eligible children not at increased risk, if Penbraya is used for dose 1 MENB, MENB-FHBP (Trumenba) should be administered for dose 2 MENB. For age-eligible children at increased risk of meningococcal disease, Penbraya may be used for additional MENACWY and MENB doses (including booster doses) if both would be given on the same clinic day and at least 6 months have elapsed since most recent Penbraya does.	
90633	HEPA VACC PED/ADOL 2 DOSE IM	Hepatitis A vaccine, Pediatric (1yo – 18yo)	
90636	HEP A/HEP B VACC ADULT IM	Hepatitis A and B vaccine, Adult (18yo ONLY)	
90647	HIB PRP-OMP VACC 3 DOSE IM	Hemophilus influenza type B vaccine (0yo – 18yo)	Pedvax
90648	HIB PRP-T VACCINE 4 DOSE IM	Hemophilus influenza type B vaccine (0yo – 18yo)	ActHIB Hibirix
90649	4VHPV VACCINE 3 DOSE IM	Human papilloma virus (HPV) vaccine, quadrivalent (9yo – 18yo)	Gardasil
90650	2VHPV VACCINE 3 DOSE IM	Human papilloma virus (HPV) vaccine, bivalent (9yo – 18yo)	Cervarix
90651	9VHPV VACCINE 2/3 DOSE IM	Human papilloma virus (HPV) vaccine, nonavalent	
90655	IIV3 VACC NO PRSV 0.25 ML IM	Influenza Virus Vaccine, Split	Fluzone
90656	IIV3 VACC NO PRSV 0.5 ML IM	Influenza Virus Vaccine, Split	
90657	IIV3 VACCINE SPLT 0.25 ML IM	Influenza Virus Vaccine, Split	Afluria Fluzone
90658	IIV3 VACCINE SPLT 0.5 ML IM	Influenza virus vaccine (3yo – 18yo)	
90660	LAIV3 VACCINE INTRANASAL	Influenza Virus Vaccine, Live	FluMist
90661	CCIIV3 VAC ABX FR 0.5 ML IM	Influenza Virus Vaccine, Deriv	Flucelvax
90671	PCV15 VACCINE IM	Pneumococcal 15-valent conjugate vaccine	Vaxneuvance

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CPT Procedure Code	*Procedure Description*	*Lay Term Description*	*Brand Name Example(s)*
90672	LAIV4 VACCINE INTRANASAL	Influenza virus vaccine, quadrivalent; for intranasal use (2yo – 18yo)	FluMist
90674	CCIIV4 VAC NO PRSV 0.5 ML IM	Influenza virus vaccine, quadrivalent; for intramuscular use (6mo – 18yo)	Flucelvax
90677	PCV20 VACCINE IM	Pneumococcal 20-valent conjugate vaccine	Prevnar-20
90678	RSV VACC PREF BIVALENT IM	Respiratory Syncytial Virus (RSV) vaccine	Abrysvo
90680	RV5 VACC 3 DOSE LIVE ORAL	Rotavirus vaccine, pentavalent (0yo – 1yo)	
90681	RV1 VACC 2 DOSE LIVE ORAL	Rotavirus vaccine (0yo – 1yo)	Rotarix
90682	RIV4 VACC RECOMBINANT DNA IM	Influenza virus vaccine, quadrivalent	
90685	IIV4 VACC NO PRSV 0.25 ML IM	Influenza virus vaccine, quadrivalent; for intramuscular use (~6mo – 2yo)	Fluzone Afluria
90686	IIV4 VACC NO PRSV 0.5 ML IM	Influenza virus vaccine, quadrivalent; for intramuscular use	Fluarix Flulaval Fluzone Afluria
90687	IIV4 VACCINE SPLT 0.25 ML IM	Influenza virus vaccine, quadrivalent; for intramuscular use (~6mo – 2yo)	Fluzone Afluria
90688	IIV4 VACCINE SPLT 0.5 ML IM	Influenza virus vaccine, quadrivalent; for intramuscular use	Fluzone Afluria
90696	DTAP-IPV VACCINE 4-6 YRS IM	Diphtheria, tetanus, acellular pertussis, and polio vaccine (4yo – 6yo)	Kinrix
90697	DTAP-IPV-HIB-HEPB VACCINE IM	Diphtheria, tetanus, acellular pertussis, polio, haemophilus influenzae type B, and hepatitis B vaccine	Vaxelis
90698	DTAP-IPV/HIB VACCINE IM	Diphtheria, tetanus, acellular pertussis, polio, and haemophilus influenzae type B vaccine (0yo – 4yo)	Pentacel
90700	DTAP VACCINE < 7 YRS IM	Diphtheria, tetanus, and acellular pertussis vaccine (0yo – 6yo)	
90702	DT VACCINE UNDER 7 YRS IM	Diphtheria and tetanus vaccine (0yo – 7yo)	
90707	MMR VACCINE SC	Measles, mumps, and rubella vaccine (1yo – 18yo)	
90710	MMRV VACCINE SC	Measles, mumps, rubella, and varicella vaccine (4yo – 12yo)	
90713	POLIOVIRUS IPV SC/IM	Polio virus vaccine	

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CPT Procedure Code	*Procedure Description*	*Lay Term Description*	*Brand Name Example(s)*
90714	TD VACC NO PRESV 7 YRS+ IM	Diphtheria and tetanus vaccine (7yo – 18yo)	
90715	TDAP VACCINE 7 YRS/> IM	Diphtheria, tetanus, and acellular pertussis vaccine (7yo – 18yo)	
90716	VAR VACCINE LIVE SUBQ	Varicella vaccine (1yo – 18yo)	
90723	DTAP-HEP B-IPV VACCINE IM	Diphtheria, tetanus, acellular pertussis, hepatitis B, and polio vaccine (0yo – 18yo)	Pediarix
90732	PPSV23 VACC 2 YRS+ SUBQ/IM	Pneumococcal vaccine, polyvalent (2yo – 18yo)	Pneumovax 23
90733	MPSV4 VACCINE SUBQ	Meningococcal Polysaccharide	Menomune
90734	MENACWYD/MENACWYCRM VACC IM	Meningococcal vaccine, serogroups A, C, W, Y, diphtheria toxoid carrier vaccine	
90744	HEPB VACC 3 DOSE PED/ADOL IM	Hepatitis B vaccine	
90756	CCIIV4 VACC ABX FREE IM	Influenza virus vaccine, quadrivalent	
91304	SARSCOV2 VAC 5MCG/0.5ML IM	COVID-19 vaccine	
91318	SARSCOV2 VAC 3MCG TRS-SUC IM	COVID-19 vaccine	
91319	SARSCV2 VAC 10MCG TRS-SUC IM	COVID-19 vaccine	
91320	SARSCV2 VAC 30MCG TRS-SUC IM	COVID-19 vaccine	
91321	SARSCOV2 VAC 25 MCG/.25ML IM	COVID-19 vaccine	
91322	SARSCOV2 VAC 50 MCG/0.5ML IM	COVID-19 vaccine	

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A.7.4 ImmPRINT Immunization Provider Registry

The Alabama Department of Public Health has established a statewide lifespan immunization registry. Effective September 1, 2018, all VFC providers are required to submit all VFC vaccines administered for accountability. Please visit their website at https://siis.state.al.us/ImmPrint/login/login.aspx for more information.

A.7.5 Recommended Immunization Schedule

You may access the recommended immunization schedule at https://www.cdc.gov/vaccines/schedules/.

The schedule indicates the recommended ages for routine administration of currently licensed childhood vaccines. Combination vaccines may be used whenever any components of the combination are indicated and its other components are not contraindicated. Providers should consult the manufacturers' package inserts for detailed recommendations.

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A.8 Required Screening Protocols

The following tables list screening protocols for infants, children, and adolescents by recipient age.

	Infancy							Early Childhood					
Age[1]	**3-5 d[2]**	**1 Mo**	**2 Mo**	**4 Mo**	**6 Mo**	**9 Mo**	**12 Mo**	**15 Mo**	**18 Mo**	**24 Mo**	**30 Mo**	**3 Yr**	**4 Yr**
History and Physical Exam[3]	X	X	X	X	X	X	X	X	X	X	X	X	X
Measurements													
Length/Height and Weight	X	X	X	X	X	X	X	X	X	X	X	X	X
Head Circumference	X	X	X	X	X	X	X	X	X	X			
Weight for Length	X	X	X	X	X	X	X	X	X				
Body Mass Index (BMI)[4]										X	X	X	X
Blood Pressure/Pulse	S	S	S	S	S	S	S	S	S	S	S	X	X
Sensory Screening													
Vision	S	S	S	S	S	S	S	S	S	S	S	X	X
Hearing	X[5]	---	---	S	S	S	S	S	S	S	S	S	X
Developmental/Social/Behavior													
Maternal Depression Screening[6]		X	X	X	X								
Developmental Screening[7]						X			X		X		
Autism Spectrum Disorder Screening[8]									X	X			
Developmental Surveillance	X	X	X	X	X		X	X		X		X	X
Behavioral/Social/Emotional Screening[9]	X	X	X	X	X	X	X	X	X	X	X	X	X
Procedures													
Newborn blood[10]	X-	---	---										
Immunization[11]	X	X	X	X	X	X	X	X	X	X	X	X	X
Anemia[12]				S			X	S	S	S	S	S	S
Lead[13]					S	S	X		S	X		S	S
Dyslipidemia[14]										S			S

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Hepatitis B Virus Infection[15]	S--	-----	-----	-----	--------	------	-------	-------	------	-------	--------	------	------
Tuberculosis[16]		S			S		S			S		S	S
Age[1]	3-5d[2]	1 Mo	2 Mo	4 Mo	6 Mo	9 Mo	12 Mo	15 Mo	18 Mo	24 Mo	30 Mo	3 Yr	4 Yr
Oral Health[17]					X	X	S		S	S	S	S	S
Fluoride Varnish[18]					------	-----	-----	------	-X-	------	-----	------	------
Fluoride Supplementation[19]					S	S	S		S	S	S	S	S

Middle Childhood

Age[1]	**5 Yr**	**6 Yr**	**7 Yr**	**8 Yr**	**9 Yr**	**10 Yr**
History and Physical Exam[3]	X	X	X	X	X	X
Measurements						
Lenth/Height and Weight	X	X	X	X	X	X
Head Circumference						
Weight for Length						
Body Mass Index (BMI)[4]	X	X	X	X	X	X
Blood Pressure	X	X	X	X	X	X
Sensory Screening						
Vision	X	X	S	X	S	X
Hearing	X	X	S	X	S	X
Developmental/Social/Behavior						
Developmental Screening						
Developmental Surveillance	X	X	X	X	X	X
Behavioral/Social/Emotional Screening[9]	X	X	X	X	X	X
Procedures						
Immunization[11]	X	X	X	X	X	X
Anemia[12]	S	S	S	S	S	S
Lead[13]	S	S				
Dyslipidemia[14]		S		S	----------	----X----

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Hepatitis B Virus Infection[15]	----------	----------	----------	----------	----------	----------
Tuberculosis[16]	S	S	S	S	S	S
Oral Health[17]	S	S				
Fluoride Varnish[18]	----------					
Fluoride Supplementation[19]	S	S	S	S	S	S

Adolescent

Age[1]	**11 Yr**	**12 Yr**	**13 Yr**	**14 Yr**	**15 Yr**	**16 Yr**	**17 Yr**	**18 Yr**	**19 Yr**	**20 Yr**
History and Physical Exam[3]	X	X	X	X	X	X	X	X	X	X
Measurements										
Length/Height and Weight	X	X	X	X	X	X	X	X	X	X
Body Mass Index[4]	X	X	X	X	X	X	X	X	X	X
Blood Pressure	X	X	X	X	X	X	X	X	X	X
Sensory Screening										
Vision	S	X	S	S	X	S	S	S	S	S
Hearing	----	-----	-X[26]-	-----	---	-X[26]-	---	----	-------	--X[26]
Developmental/Social/Behavior										
Developmental Surveillance	X	X	X	X	X	X	X	X	X	X
Behavioral/Social/Emotional Screening[9]	X	X	X	X	X	X	X	X	X	X
Tobacco, Alcohol, or Drug use Assessment[20]	S	S	S	S	S	S	S	S	S	S
Depression and Suicide Risk Screening[21]		X	X	X	X	X	X	X	X	X
Procedures										
Immunization[11]	X	X	X	X	X	X	X	X	X.	X
Anemia[12]	S	S	S	S	S	S	S	S	S	S
Dyslipidemia[14]	----	S	S	S	S	S	------	------	----X-----	-------
Hepatitis B Virus Infections[15]	-----	-----	------	------	----	------	-------	------	------	--------
Tuberculosis[16]	S	S	S	S	S	S	S	S	S	S

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Fluoride Supplementation[19]	S	S	S	S	S	S				
Sexually Transmitted Infections[22]	S	S	S	S	S	S	S	S	S	S
Sudden Cardiac Arrest/Death[23]	S--	-----	-----	-----	----	-----	-----	------	--------.	--------
Hepatitis C Virus Infection[24]								X---	--------	---------
HIV[25]	S	S	S	S	X--	------	-------	------	----------	---------

Anticipatory Guidance	Anticipatory guidance is designed to ensure that children receive the necessary health care to support their growth and development. It includes screening, assessments, and anticipatory health care visits that are recommended at each age.
Urine Screening	Urine screening needs to be performed only when clinically indicated and an appropriate medical diagnosis is required.

Key:

X	Required at the visit for this age
S	Subjective assessment by history and/or observation with appropriate action to follow
---- -----	Range during which a service may be provided.
S or X	Perform risk assessments or screenings as appropriate, based on universal screening requirements.

Footnotes:

1	If a child comes under care for the first time at any point of the schedule, or if any components are not accomplished at the recommended age, the schedule should be brought up to date at the earliest possible time.
2	Newborns should have an evaluation within 3 to 5 days of birth and within 48 to 72 hours after discharge from the hospital to include evaluation for feeding and jaundice
3	The physical examination/assessment must include an oral/dental inspection. At each visit, age-appropriate physical examination is essential, with infant totally unclothed and older children undressed and suitably draped.
4	Pediatricians and other PHCPs should measure height and weight calculate BMI, and assess BMI percentile using age- and sex-specific CDC growth charts or growth charts for children with severe obesity at least annually for all children 2 to 18 years of age to screen for overweight (BMI >/= 85th percentile to <95th percentile), obesity (BMI >/=95th percentile), and severe obesity (BMI >/=120% of the 95th percentile for age and sex).
5	Confirm initial screen was accomplished, very results and follow up, as appropriate.
6	Mothers should receive screening for Perinatal depression (PND) due to the depression affecting the entire family.
7	The administration of a standardize developmental screening test is completed at the 9-, 18-, and 30-month visits**.**
8	Standardized screening for autism spectrum disorder (ASD) at 18- and 24-months of age with ongoing developmental surveillance is recommended which increase interventions that may improve function. ASD can be diagnosed as early as 18 months.
9	Screening for behavioral and social-emotional problems in children and adolescents should be family centered and may include asking about caregiver emotional and mental health concerns and social determinants of health, racism, poverty, and relational health.

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10	Confirm initial newborn screen was completed, verify results, and follow up, as appropriate.
11	Every visit should be an opportunity to update and complete a child's immunizations.
12	Perform risk assessment or screening, as appropriate, per recommendations in the current edition of the AAP Pediatric Nutrition: Policy of the American Academy of Pediatrics (Iron chapter). At 12 months a Hemoglobin and/or Hematocrit should be obtained.
13	All children are considered at risk and must be screened for lead poisoning. A blood lead test is required at 12 and 24 months of age. A lead toxicity screening is also required for any child 36 to 72 months of age who has not previously received a blood lead toxicity screening or who presents with symptoms of possible lead poisoning.
14	Provide risk assessments if positive family history for dyslipidemia, obtain fasting lipid. If no family history, obtain fasting lipid provide between ages 9y – 11y and 17y – 20y.
15	Perform a risk assessment for Hepatitis B virus (HBV) infection according to recommendations per the USPSTF and AAP making effort to preserve the confidentiality of the recipient.
16	Tuberculosis testing per recommendations of the AAP Committee on Infectious Diseases, published in the current edition of the AAP Red Book: Report of the Committee on Infectious Diseases. Testing should be performed on recognition of high-risk factors.
17	Assess whether the child has a dental home. If no dental home is identified, perform a risk assessment and refer to a dental home. Recommend brushing with fluoride toothpaste in the proper dosage for age.
18	The USPSTF recommends that the primary care clinicians apply fluoride varnish to the primary teeth of all infants and children starting at the age of primary tooth eruption.
19	If primary water source is deficient in fluoride, consider oral fluoride supplementation.
20	Screen for the use of alcohol, tobacco and nicotine, marijuana, and other substances, including opioids in adolescents.
21	Screen adolescents for depression and suicide risk, making every effort to preserve confidentiality of the adolescent.
22	Adolescents should be screened for sexually transmitted infections (STIs) per recommendations by AAP.
23	Perform a risk assessment as appropriate. The role to the primary care provider is of paramount importance in the evaluation of children, particularly as they enter middle school or junior high for sudden cardiac death.
24	All individuals should be screened for Hepatitis C Virus infection according to the USPSTF and Centers for Disease Control and Prevention recommendations at least once between the ages of 18 and 79. Those at increased risk for HCV infection, including those who are persons with past or current injection drug use, should be tested for HCV infection and reassessed annually.
25	Screen adolescents for HIV at least once between the ages of 15 and 20, making every effort to preserve confidentiality of the adolescent. After initial screening, youth at increased risk of HIV infection should be retested annually or more frequently as recommended by AAP.
26	Screen with audiometry including 6,000 and 8,000 HZ high frequencies once between 11 and 14 years, once between 15 and 17 years, and once between 18 and 20 years.

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